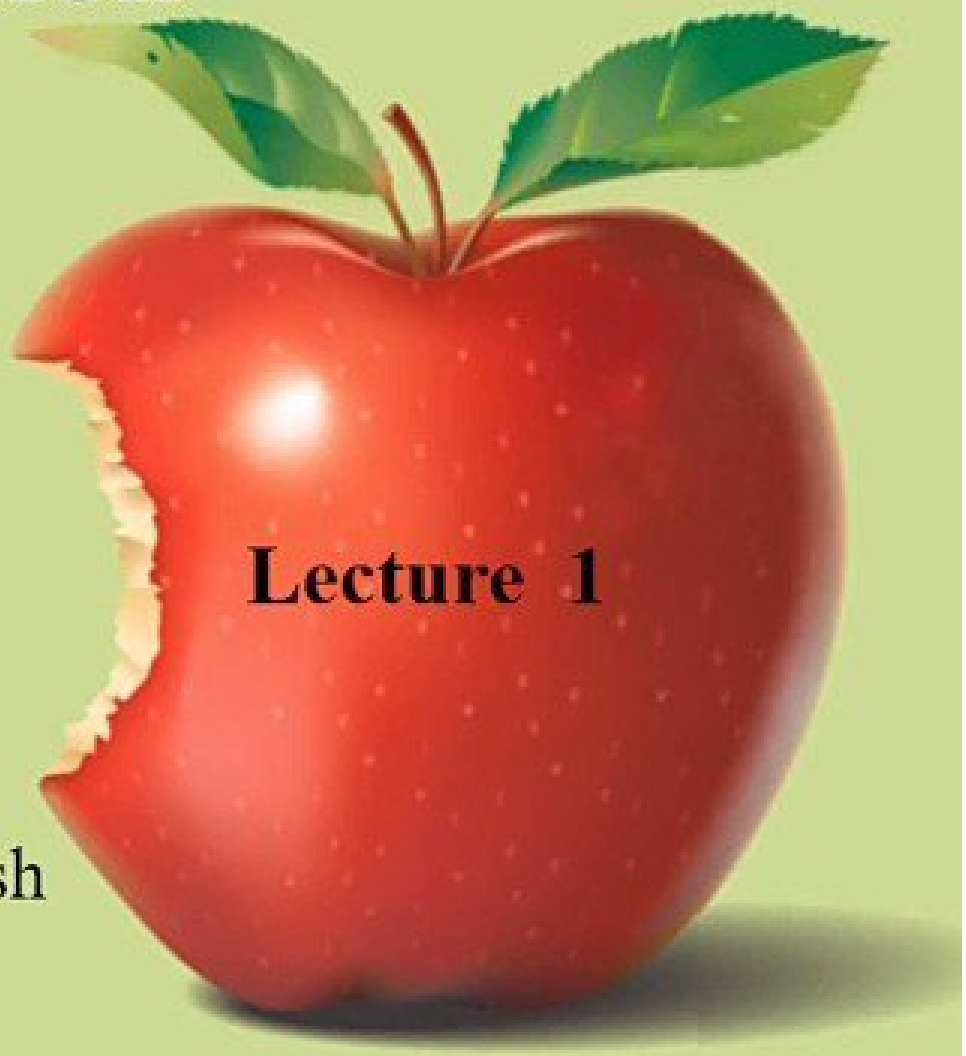
Nursing Theory Utilization & Application



Dr. Hala Aiash

2018/2019

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The Nature of Knowledge Needed for Nursing Practice

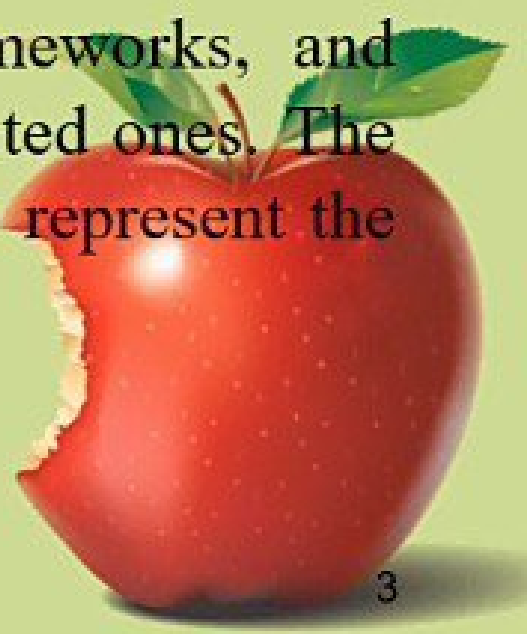
The systematic accumulation of knowledge is essential to progress in any profession… however, theory and practice must be constantly interactive. Theory without practice is empty and practice without theory is blind. (Cross, 1981, p.110)



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This course first addresses basic ideas about the meaning of nursing theory and its relevance to professional nursing. Issues related to the purpose, use, and diversity of nursing theories are discussed. It then presents a broad overview of selected nursing theories. The major ideas of selected nursing theories are explained and examples of their use in nursing situations are provided.

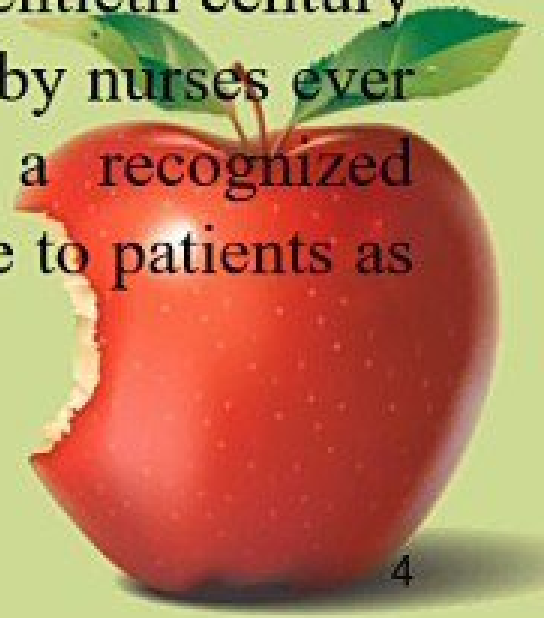
Although there are many nursing theories, frameworks, and models in nursing, this course addresses only selected ones. The theories discussed have been selected because they represent the development of nursing's scientific thought.



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Nursing has made phenomenal achievement in the last century that has led to the recognition of nursing as an academic discipline and a profession. A move towards theory-based practice has made contemporary nursing more meaningful and significant by shifting nursing's focus from vocation to an organized profession.

The need for knowledge-base to guide professional nursing practice had been realized in the first half of the twentieth century and many theoretical works have been contributed by nurses ever since, first with the goal of making nursing a recognized profession and later with the goal of delivering care to patients as professionals

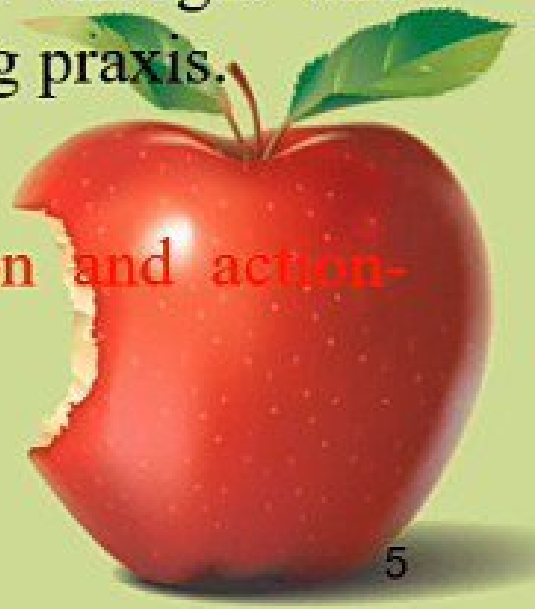


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In this twenty-first century, many nurses understand the vital role of conceptual and theoretical knowledge structures for quality nursing practice and nursing research.

We have theoretical structures that provide nurses with a patient focus and frameworks to identify and interpret patient data for evidence based practice. Where as nursing had been understood as an art and a science, the theory era strengthened understanding of the vital nature of coordinated thought and action in practice now referred to by many a nursing praxis.

(Praxis is descriptive of knowledge-guided action and action-guided theory)



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Eras of Nursing Knowledge

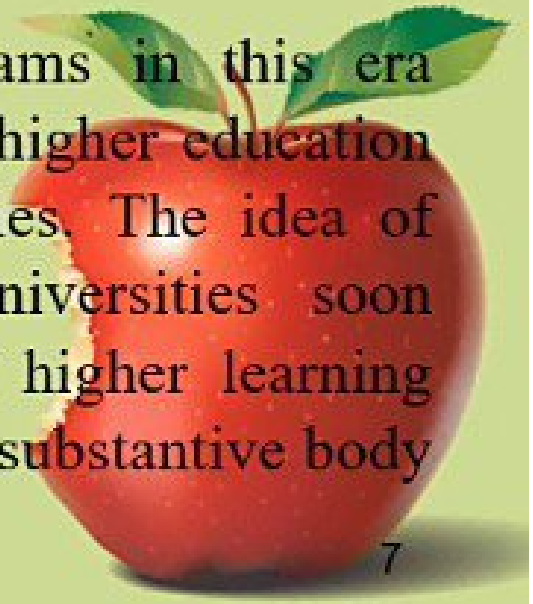
- Signs of a national consciousness for nursing may be seen in the first national gathering of nurses at the World's Fair in Chicago in 1893 and in the publication of the first edition of the American Journal of Nursing (AJN), the first national organ of communication for nurses, in October 1900 (Kalisch & Kalisch, 2004).
- At this early time the focus was clearly on practice and on teaching the practice of nursing to students. There was recognition of the need for specialized knowledge to guide the practice of nursing from the beginning.
- AJN was one early symbol of nursing's movement toward professional status, and another was their need to communicate with other nurses about their practice and about teaching nursing.



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Curriculum Era: The 1900s to the 1940s

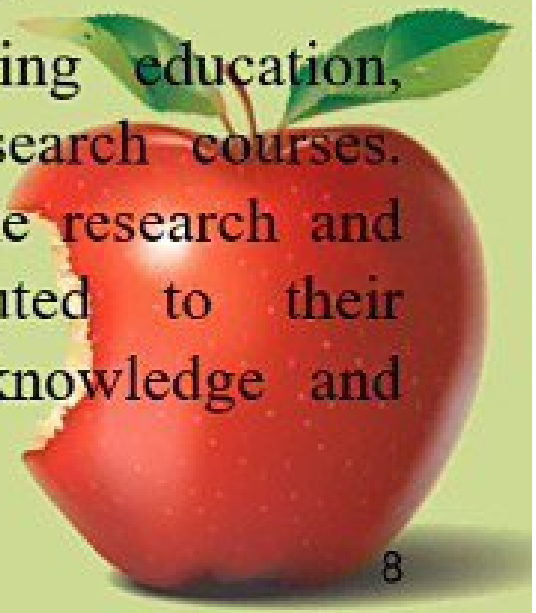
- The focus of this era was evident in state activities such as the 1933 curriculum survey of New York training schools. This emphasis on what nurses needed to know to practice nursing led to an expansion of curricula beyond physiological and pathophysiological knowledge to include social sciences, pharmacology, and formal classes on nursing Procedures.(*which called fundamentals, a term that means "basic essentials)*
- Nursing curricula taught mostly in diploma programs in this era became standardized and some nurses began to seek higher education courses related to nursing in colleges and universities. The idea of developing nursing programs in colleges and universities soon followed. The transition of nursing into schools of higher learning brought with it a significant change in the search for a substantive body of knowledge.



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Research Era: The 1950s and the 1970s

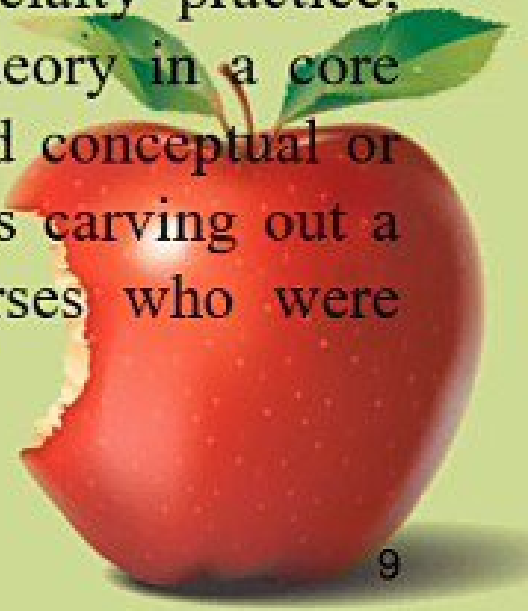
- This era saw the development of scholarship and the dissemination of early research findings. *Nursing Research,* the first nursing research journal, was established for this purpose in 1952. In addition, two programs funded by the federal government were instituted in 1955 to prepare nurses as researchers and teachers of research—the U.S. Public Health Service predoctoral research fellowships and the Nurse Scientist Training program.
- The developments in research influenced nursing education, emphasizing graduate education with nursing research courses. Emphasis on faculty nursing scholarship during the research and graduate education eras undoubtedly contributed to their interrelationship, as well as the close ties of knowledge and research.



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Graduate Education Era: The 1950s and the 1970s

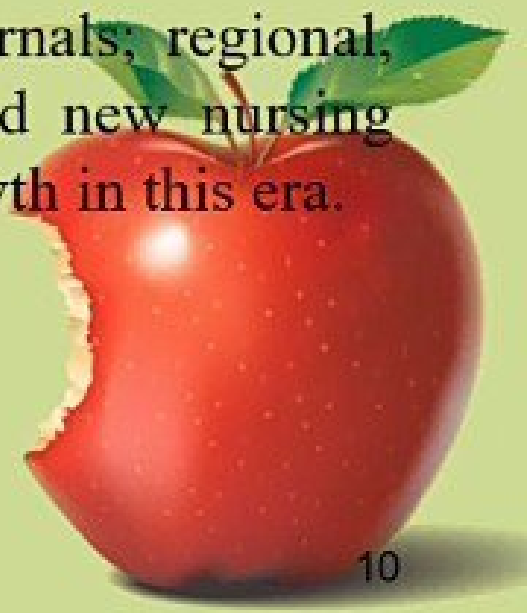
- During the graduate education era, curricula for master's-level preparation were becoming standardized through accreditation that most schools were seeking by the National League for Nursing (NLN). Nurse educators came together at national meetings where accreditation criteria were approved.
- By the end of the 1970s, most accredited master's programs included courses in nursing research, clinical specialty practice, leadership, and concept development or nursing theory in a core curriculum organized with a nursing philosophy and conceptual or organizing framework. A major task of this era was carving out a role in health care for the master's-prepared nurses who were graduating.



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Theory Era: The 1980s and 1990s

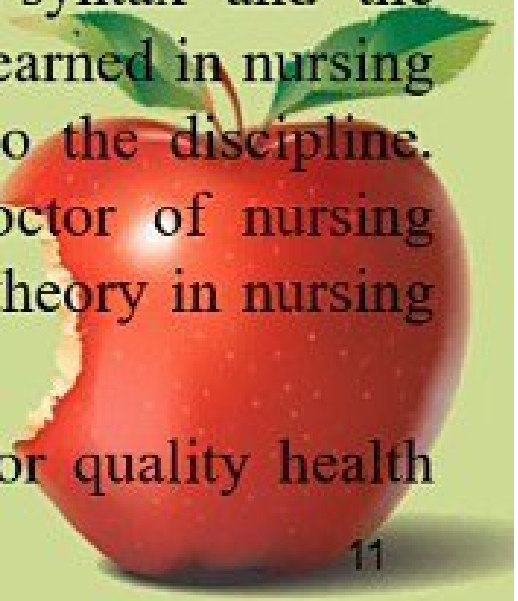
- The theory era began with a strong emphasis on knowledge Development. The theory era, coupled with the research and graduate education eras, led to understanding of the scientific process beyond production of a scientific product
- First editions of several nursing theory texts in this era included contemporary nursing theorists.
- Proliferation of nursing literature; new nursing journals; regional, national, and international nursing conferences; and new nursing doctoral programs were evidence of exponential growth in this era.



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Theory Utilization Era: The Twenty-First Century

- Nursing is now in the era of theory utilization—nurses using philosophies, models, and theories for theory-based nursing practice. Soon after we entered the twenty-first century sufficient evidence of theory-based practice existed to declare a theory utilization era .This important era in the history of nursing continued to thrive and be recognized as vital to nursing's future.
- Theory development with analysis and critique of syntax and the structure of theory is how knowledge development is learned in nursing doctoral programs, especially PhD, which are vital to the discipline. Theory courses in practice-focused master's and doctor of nursing practice (DNP) programs focus on the application of theory in nursing practice.
- Theory utilization is in concert with national goals for quality health care.



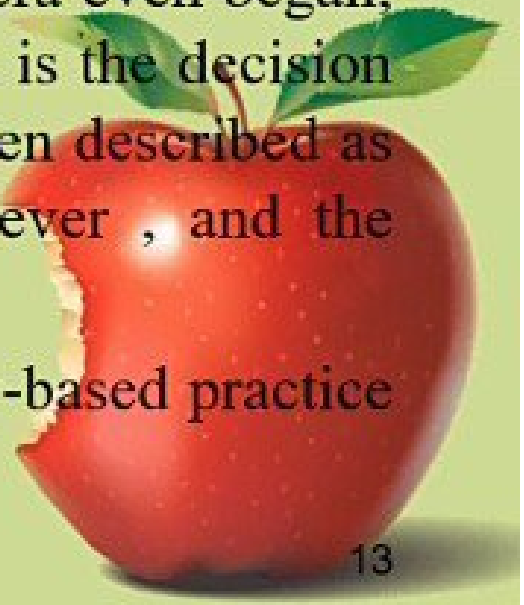
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Table :Summary of Historical Eras of Nursing's Search for Specialized Knowledge

Historical Eras	Major Question	Emphasis	Outcomes	Emerging Goal
Curriculum era: 1900 to the 1940s	What curriculum content should student nurses study?	Courses taught in nursing programs	Standardized curricula for diploma programs	Specialized knowledge and higher education
Research era: 1950 to the 1970s	What is focus for nursing research?	Role for nurses and what to research	Problem studies and studies of nurses	Theory-based studies for unified knowledge
Graduate education era: 1950 to the 1970s	What knowledge is needed for nursing practice?	Carving out an advanced role and basis for nursing practice	Nurses have an important role in quality health care	Focus graduate education on knowledge development
Theory era: 1980 to the 1990s	How do these frameworks guide research and practice?	There are many ways to think about nursing	Nursing theoretical works clearly focus on the patient	These theories guide nursing research and practice
Theory utilization era: 21st century	What new theories are needed as evidence for quality care?	Nursing theory guides research, practice, education, and administration	Middle-range theories are from quantitative or qualitative approaches	Nursing frameworks are the knowledge (evidence) for quality care

Centrality of Nursing Practice

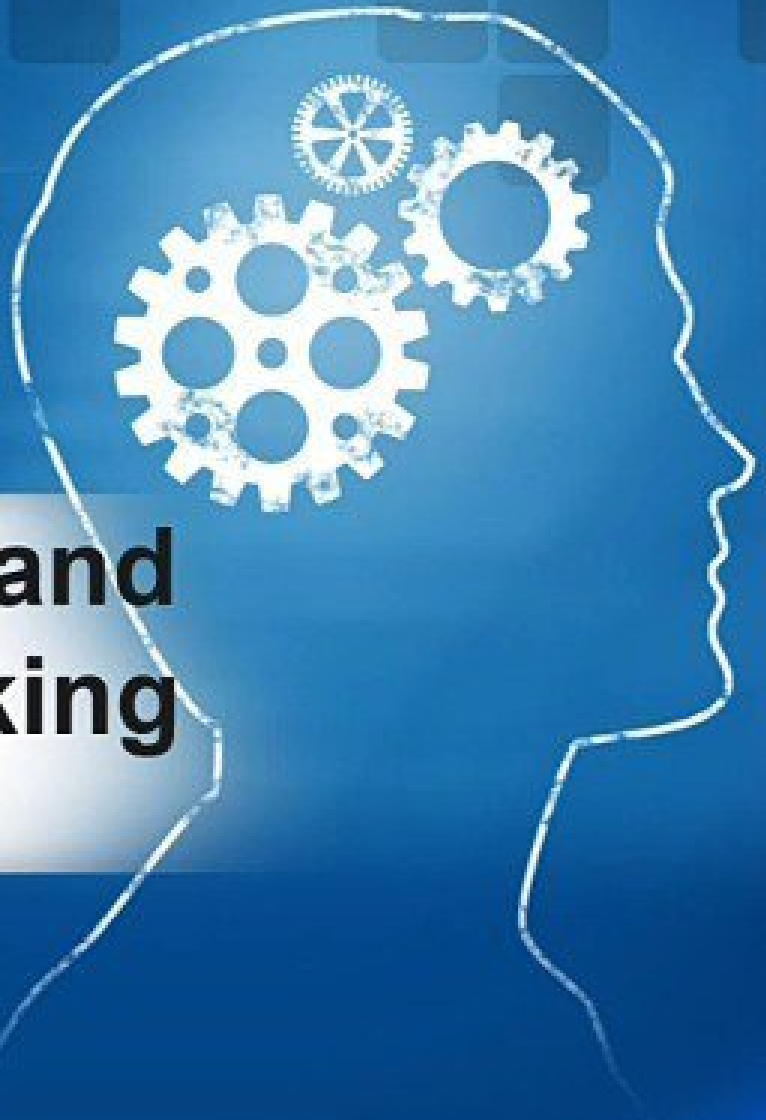
- The centrality of practice to nursing is vital to understand the purpose of theory. It is vital to progress in the discipline for nurses to use nursing practice approaches in spite of pressure to embrace the biophysical view of medicine for their practice. Continued progress depends on demonstrations of quality outcomes with nursing perspectives.
- Mathwig (1975) noted very early, before the theory era even began, that the first phase of translating theory into practice is the decision to do so. Similarly, use of a model in practice has been described as a habit to be formed , the practice of a true believer , and the practice of one who has been properly persuaded
- Progression of nurses to theory utilization and theory-based practice is best explained by nursing history.



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Lecture
2

Philosophies, Models, and Theories: Critical Thinking Structures



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2020/2021

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It is not simply knowing a lot of things; it is a way of knowing things. *(Levine, 1988)*

"Theory development in nursing science is critical for evolution of the discipline" and growth of the profession is dependent on nurses knowing and using nursing theoretical works in their practice of nursing. Studies continue to generate evidence of the connections among knowledgeable use of nursing theoretical works, education, and quality nursing practice. This evidence is vital for the practicing nurse.

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The Relationship of Philosophies, Models, and Theories

The philosophies, models, and theories of a discipline are theoretical structures that address the central concepts of that discipline. The science of nursing is recognized as a fundamental pattern of knowing for nurses

The **metaparadigm** specifies disciplinary boundaries of human beings, environment, health, and nursing as a context to understand the interrelationships among those elements of contemporary nursing science



- **Theoretical knowledge** may be differentiated by the way it is named or labeled.
- A **model tends** to be named for the person who authors it, for example, the Neuman Systems Model.
- **Grand theories** tend to be named for the outcome they propose, for example, the Theory of Optimal Client System Stability, and
- **Theories tend** to be named for the characteristics their content demarcates as an explanatory shell of the outcomes they propose.

An example is Gigliotti's (2003) theory of women's multiple role stress, which she validated as middle range with the age groups of women and forms of stress (Gigliotti, 2011, 2012).

Table (1) A Structure of Nursing Knowledge Types and an Example of Each Type

Knowledge Type	Examples of Types of Nursing Knowledge
Metaparadigm	Human beings, environment, health, nursing
Philosophy	Nightingale's philosophy
Conceptual models	Neuman Systems Model
Grand theory	Optimal Client System Stability
Theory	Flexible line of defense moderates Optimal Client System Stability
Middle-range theory	Flexible line of defense moderates stress levels for Optimal Client System Stability in women who are enacting multiple roles

Knowledge types based on Fawcett, J. (2005). Contemporary nursing knowledge: Conceptual models and theories (2nd ed.). Philadelphia: F. A. Davis.



Table (1) presents types of nursing knowledge at each level of abstraction and an example of nursing knowledge for each type.

The metaparadigm is the most abstract set of central concepts for the discipline of nursing (i.e., human being, environment, health, nursing), and these concepts are defined within each of the conceptual models and according to the philosophy of that model.

Philosophies present the general meaning of nursing and nursing phenomena through reasoning and logical presentation of ideas. Although Nightingale (1946) did not present her philosophy on the relationship of patients and their surroundings as a theory, her philosophy contains implicit theory that guides nursing practice.

Philosophies are theoretical works that address one or more of the metaparadigm concepts (person, environment, health, and nursing) in a broad philosophical way.



Conceptual models (also called paradigms or frameworks) such as the Neuman Systems Model (Neuman & Fawcett, 2011) are the next less abstract set of concepts in the structure.

Nursing models are frameworks or paradigms of the science of nursing that address the person, environment, health, and nursing metaparadigm.

Grand theory (e.g., Neuman's Theory of Optimal Client System Stability) is next as the level of abstraction descends. Theory can be considered grand when it is nearly as abstract as the model itself and when the usefulness of the model depends on the soundness of that theory. Grand theory is especially useful in research and practice because it is more general, and theories specifying the details of practice can be derived from it.

Nursing theories derive from models and are guiding structures for reasoning and decision making about the person, the person's health situation, and the care indicated



Theory is the next less abstract level; it is more specific than grand theory but not as specific as middle-range theory (e.g., Optimal Client System Stability in specific settings).

Theories have been specified as important forms of evidence. Evidence-based nursing practice is encouraged but is dependent on ways to recognize quality evidence. And the quality of evidence-based practice is dependent on recognition of quality research

Middle-range theory is the least abstract set of concepts and the most specific to nursing practice (e.g., promoting Optimal Client System Stability through a stress reduction intervention in the work environment). The understanding of theory terminology is developed over time with knowledge of works at the different levels of abstraction (Fawcett, 2005; Reynolds, 1971).

These theories are at the practice level, and include details of nursing practice.



When a theory is at the grand theory level, many middle-range applications of that theory can be developed for practice by specifying factors such as:

- The situation or health condition
- The client population or age group
- The location or area of nursing practice (e.g., home, hospital, community)
- The action of the nurse or nursing intervention

Moral Implications of Philosophically or Theoretically Guided Practice

- A grasp of the implications of certain viewpoints gives nurses the capacity for adjusting practice in such a way as to most consistently address health for the individual, group, or society.
- The nurse has three main responsibilities if the ramifications of a given perspective seem dissonant with any, or any combination, of the following:

1) The context or situation
2) The nurse's philosophy
3) The patient's philosophy or beliefs

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Moral Implications of Philosophically or Theoretically Guided Practice…….

- First,(***The context or situation***) it is necessary to check that the framework is consistent and well developed.

- Second,(***The nurse's philosophy***) the nurse must understand whether it is being utilized as intended. An inadequate grasp of both the framework and its implications for practice has the potential for poor or inconsistent practice.

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Moral Implications of Philosophically or Theoretically Guided Practice…….

Third,(***The patient's philosophy or beliefs***) it is important to investigate whether another framework is a better fit for the setting, the patient's situation and/or beliefs about health, or the practitioner's beliefs.

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Benner's Philosophy in Nursing Practice

Dr. Hala Aiash
Samer Dahalan
Ahmed abu Shamla
2020/2021



A caring, involved stance is the prerequisite for expert, creative problem solving. This is because the most difficult problems to solve require perceptual ability as well as conceptual reasoning, and perception requires engagement and attentiveness.

(Benner, 1984, p. 214).

◆History and Background

- More than 30 years ago, Benner began what she describes as an articulation project of the knowledge embedded in nursing practice. Her initial thrust toward further understanding of the theory/practice gap in nursing became transformed while conducting the Achieving Methods of Intra-Professional Consensus, Assessment and Evaluation (AMICAE) project.

- This constituted a paradigm shift in nursing by demonstrating that knowledge can be developed in practice, not just applied, and signifying that practice is a way of knowing in its own right.

- Two direct outcomes of the AMICAE research project were:
- (1) validation and interpretation of the Dreyfus model of skill acquisition for nurses and
- (2) description of the domains and competencies of nursing practice.
- Benner's work is research based and derived from actual practice situations. Benner's work has been developed and applied in general staff nursing, critical care nursing, community health nursing, advanced practice nursing, and nursing education.
- Benner's research offers a radically different perspective from the cognitive rationalist quantitative paradigm prevalent during the 1970s and 1980s. Her research constitutes an interpretive turn—a move away from epistemological, linear, analytical, and quantitative methods toward a new direction of ontological, hermeneutic, holistic, and qualitative approaches.

◆Overview of Benner's Philosophy

- Nursing is a caring practice guided by the moral art and ethics of care and responsibility that unfolds in relationships between nurses and patients.
- The original domains and competencies of nursing practice were identified and described inductively from clinical situation interviews and observations of novice and expert staff nurses in actual practice.
- This interpretive phenomenological study used a situational approach to the study of the knowledge and meanings embedded in the everyday practice of nurses. "The strength of this method lies in identifying competencies from actual practice situations rather than having experts generate competencies from models or hypothetical situations".

These competencies were grouped according to similarities of function, intent, and meaning to form seven domains of nursing practice:

1- The helping role

2- The teaching-coaching function

3- The diagnostic and patient-monitoring function

4- Effective management of rapidly changing situations

5- Administering and monitoring therapeutic interventions and regimens

6- Monitoring and ensuring the quality of health care practices

7- Organizational and work-role competencies

- **Benner and colleagues have focused on articulating skill acquisition processes and competencies of nurses in acute and critical care areas. Domains and competencies have also been useful for articulation of knowledge embedded in advanced nursing practice.**
- **Fenton's (1985) study indicated that the original domains were present in the practice of clinical nurse specialists (CNSs). She identified additional competencies for three of Benner's original domains and described one additional domain, *the consulting role of the nurse*.**

- **Fenton described the competency *making the bureaucracy respond* in her study of CNSs. This involved knowing how and when to work around bureaucratic roadblocks in the system so patients and families could receive needed care.**

- **Brykczynski (1985) developed an additional domain from her study of outpatient nurse practitioner (NP) practice. The new domain consolidated two of Benner's domains that were typical of inpatient nursing practice—*diagnostic and patient monitoring function* and *administering and monitoring therapeutic interventions and regimens* and replaced it with *management of patient health/illness status in ambulatory care settings.* The remaining five of Benner's seven domains were interpreted as valid for the practice of the NPs studied.**

- **NPs practiced more as direct providers of care, whereas CNSs functioned more as facilitators of care. The new domain, *the consulting role of the nurse,* was evidenced in the practice of both CNSs and NPs. Competencies in this domain represent the initial articulation of skills and knowledge specific for advanced nursing practice.**

◆ Critical Thinking in Nursing Practice with Benner's Approach

- Benner addresses critical thinking in a developmental and interpretive way. Whereas formal definitions of critical thinking tend to signify abstract, rational calculation with analysis and weighing of options to arrive at decisions.

- Benner and colleagues (1999) identified six aspects of clinical judgment and skillful comportment that can be viewed as interpretive aspects of critical thinking (Table 4-1). These six aspects were identified and described through study of critical care nursing practice.

- They constitute components of the thinking-in-action approach required in acute and critical care.

Table 4-1 **Critical Thinking in Nursing Practice with Benner's Philosophy**

Aspects of Clinical Judgment and Skillful Comportment	Definition
1. Reasoning in transition	Practical reasoning in an ongoing clinical situation
2. Skilled know-how	Embodied intelligent performance, which involves knowing what to do, when to do it, and how to do it
3. Response-based practice	Adapting interventions to meet changing needs and expectations of patients
4. Agency	One's sense of and ability to act on or influence a situation
5. Perceptual acuity and involvement	Ability to tune in to a situation and hone in on the salient issues by engaging with the problem(s) and the person(s)
6. Links between clinical and ethical reasoning	Understanding that good clinical practice cannot be separated from ethical notions of good outcomes for patients and families

Developed from Benner, P., Hooper-Kyriakidis, P., & Stannard, D. (1999). *Clinical wisdom and interventions in critical care: A thinking-in-action approach*. Philadelphia: Saunders.

- **Benner has suggested that both practical clinical knowledge and caring practices have been minimized in modern health care. It is her hope that identification and description of these practices on which health care institutions depend will promote their recognition and legitimacy. These aspects of practice have particular relevance to minimizing errors and promoting safety in health care situations (Benner, Malloch, & Sheets, 2010).**
- **In Benner's work nursing practice is an interpretive (hermeneutic) of the patient situation. She agrees with Good and Good (1981) that "all clinical encounters have a hermeneutic dimension; clinicians and patients interpret one another's meanings to bring to light an underlying coherence or sense".**
- **For Benner, nursing practice is constituted by a circular hermeneutic process between the nurse, the patient, and the ongoing situational context.**

Nursing Theory Utilization & Application

Lecture 3

Dr. Hala Aiash

2020-2021



Application

Utilization of nursing theoretical works illustrates their explicit focus and applicability as frameworks for evidence-based nursing practice and quality care.

1- Philosophies

- Nightingale application focuses on patients and their environmental surroundings
- Watson application uses caritas processes to guide caring nursing actions
- Benner application provides care in a healing nurse and patient relationship



Nightingale application focuses on patients and their environmental surroundings



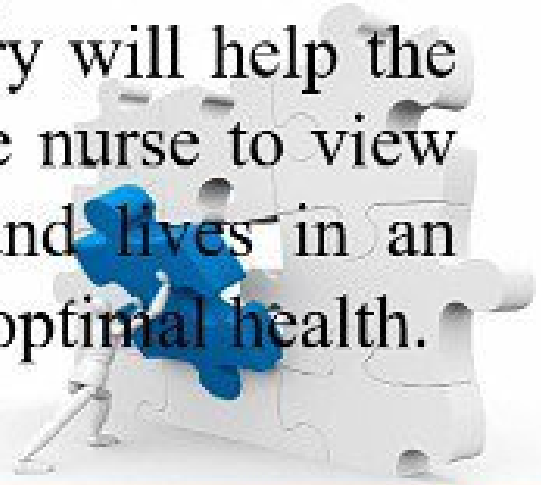


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I use the word nursing for want of a better. It has been limited to signify little more than the administration of medicine and application of poultices. It ought to signify the proper use of fresh air, light, warmth, cleanliness, quiet, and the proper selection and administration of diet—all at the least expense of vital power to the patient. (Nightingale, 1969, p. 8)



Nightingale's environmental theory arose out of a need to reform nursing and the environment of England in the late 1800s. Her theory is simple and easy to apply to nursing practice because it contains many of the same elements that nursing students learn during their education. Use of Nightingale's theory will help the nurse have a beginning focal point and allows the nurse to view the client as an individual who interacts with and lives in an environment that may or may not be conducive to optimal health.



History and Background

- Nightingale was born in 1820 in Florence, Italy. Her parents were very wealthy and often traveled abroad.
- At the age of 24, Nightingale decided she needed to help the suffering masses and wished to work in a hospital. This was met with opposition from her family, and they fought about it for years before finally allowing her to go to Kaiserworth, Germany, to learn nursing from the Institution of Deaconesses She studied there for 3 months and then returned to the service of her family.
- She developed what we have come to refer to as her nursing theory after her travel to Scutari to care for wounded soldiers during the Crimean War. Her writings, which included philosophy and directives, were developed from a need to define nursing and reform hospital environments rather than for the purpose of providing nursing new Knowledge.

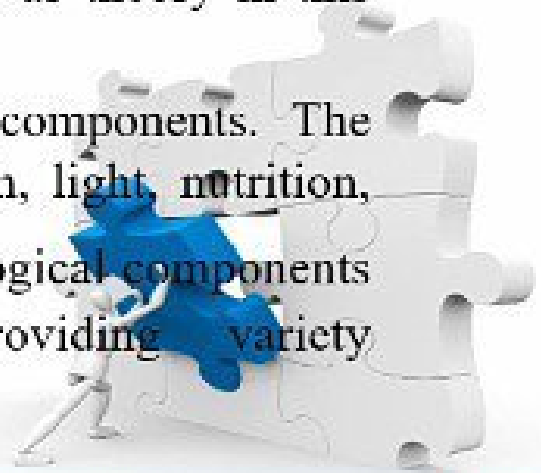


Overview of Nightingale's Environmental Philosophy

Nightingale's philosophy is environmentally oriented. She believed that the environment of the patient should be altered to allow nature to act on the patient.

The components of Nightingale's philosophy, which is now recognized as theory in this theory era, are the following:

- **Environment:** This theory has both physical and psychological components. The physical components of the environment refer to ventilation, warmth, light, nutrition, medicine, stimulation, room temperature, and activity . The psychological components include avoiding chattering hopes and advices and providing variety



- **Person**: Although most of Nightingale's writings refer to the person as the one who is receiving care, she did believe that the person is a dynamic and complex being.

- **Health:** Nightingale (1954b) wrote, "Health is not only to be well, but to be able to use well every power we have" (p. 357). From this statement we can infer that she believed in prevention and health promotion in addition to nursing patients from illness to health.

- **Nursing:** Nightingale believed nursing to be a spiritual calling. Nurses were to assist nature that was healing the patient



Critical Thinking Using Nightingale's Theoretical Philosophy

The term ***critical thinking*** was not in use in Nightingale's day; however, she expected nurses to use their powers of observation in caring for patients. he wanted her nurses "to be clear thinkers and independent in their judgments" She advocated for nurses to have educational backgrounds and knowledge that were different from those of physicians.

Table 5-1 illustrates the interaction of the nurse and the patient with the use of Nightingale's environmental theory.

Because Nightingale believed in nursing well persons—or health promotion—it is logical that she assumed her nurses would complete some health teaching as they were caring for the sick or for those who were already well.



TABLE 5-1 Critical Thinking with Nightingale's Theory

Nightingale's Canons (Nightingale, 1969; Selanders, 1998)	Nursing Process and Thought
Ventilation and warmth	Assess the client's body temperature, room temperature, and room for fresh air (or adequate ventilation) and foul odors. Develop a plan to keep the room airy and free of odor while maintaining the client's body temperature.
Light	Assess the room for adequate light. Sunlight works best. Develop and implement adequate light in the client's room without placing the client in direct light.
Cleanliness of rooms and walls	Assess the room for dampness, darkness, and dust or mildew. Keep the room free from dust, dirt, mildew, and dampness.
Health of houses	Assess the surrounding environment for pure air, pure water, drainage, cleanliness, and light. Examples include removing garbage or garments from the area, removing any standing water (or ensuring that water drains away from the area), and ensuring that air and water are clean and free from odor and that there is plenty of light.
Noise	Assess the noise level in the client's room and surrounding area. Attempt to keep noise level to a minimum, and refrain from whispering outside the door.
Bed and bedding	Assess the bed and bedding for dampness, wrinkles, and soiling, and check the bed for height. Keep the bed dry, wrinkle-free, and at the lowest height to ensure the client's comfort.
Personal cleanliness	Attempt to keep the client dry and clean at all times. Frequent assessment of the client's skin is needed to maintain adequate skin moisture.
Variety	Attempt to stimulate variety in the room and with the client. This is accomplished with cards, flowers, pictures, books, or puzzles. Encourage friends and relatives to engage the client in some sort of stimulating conversation.
Chattering hopes and advices	Avoid talking without reason or giving advice that is without fact. Continue to talk to the client as a person, and continue to stimulate the client's mind. Avoid personal talk.
Taking food	Assess the diet of the client. Take note of the amount of food and drink ingested by the client at every meal or snack.
What food	Continue with the assessment of the diet to include type of food and drink the client likes or dislikes. Attempt to ensure that the client always has some food or drink available that he or she enjoys.
Petty management	Petty management ensures continuity of care. Documentation of the plan of care and all evaluation will ensure others give the same care to the client in your absence.
Observation of the sick	Observe everything about your client. Record all observations. Observations should be factual and not merely opinions. Continue to observe the client's surrounding environment, and make alterations in the plan of care when needed.

Nursing Theory Utilization & Application

Models

Dr. Hala Aiash
2020-2021



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Nursing conceptual models (or frameworks) provide comprehensive perspectives for nursing practice. Models are broad conceptual structures that provide holistic views and specific foci of nursing. They are organizing frameworks for a particular perspective of nursing practice, such as adaptation, person-environmental process, interaction, or self-care. This section reviews seven works that have been specified as nursing models by analysis and evaluation

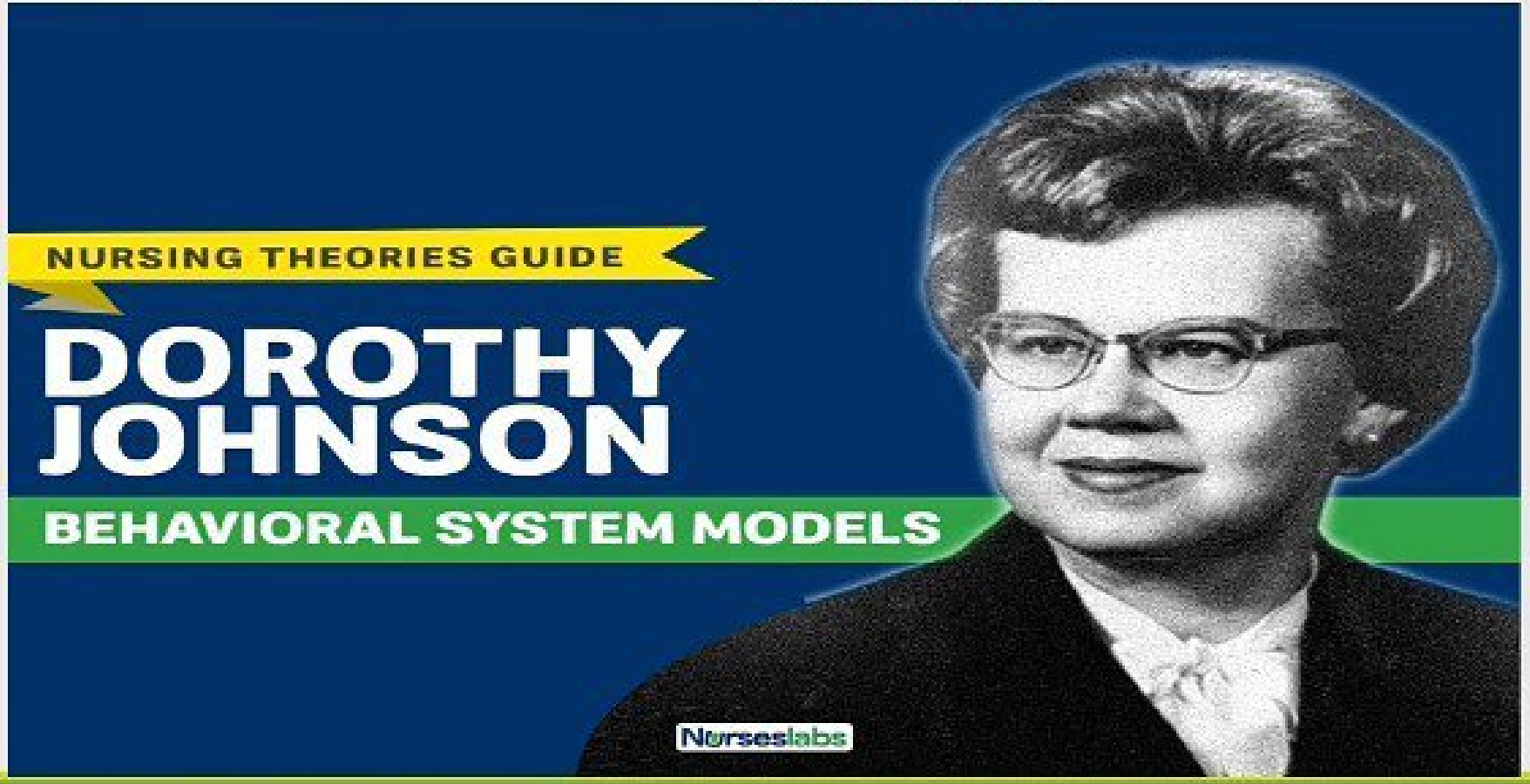


- Johnson application bases nursing care on a systematic view of patient behavior
- King application uses a conceptual system and theory for goal attainment
- Levine application relates conservation principles to adaptive health promotion
- Neuman application is nursing from a systematic view of strengths and stressors
- Orem application of self-care defcit theory identifes self-care agency and needs
- Rogers application is quality care from human/environmental feld patterning
- Roy application uses environmental assessment and adaptive modes to plan care



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Johnson's Behavioral System Model in Nursing Practice



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Nursing is "an external regulatory force that acts to preserve the organization and integration of the patient's behavior at an optimal level under those conditions in which the behavior constitutes a threat to physical or social health or in which illness is found." The goal of nursing is to "restore, maintain or attain behavioral integrity, system stability, adjustment and adaptation, efficient and effective functioning of the system.

(Johnson, 1980, p. 214)



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History and Background

The Johnson Behavioral System Model (JBSM) was conceived and developed by Dorothy Johnson while she was a professor of nursing at the University of California, Los Angeles (UCLA).

The process of developing this model began in the late 1950s as she examined the explicit goal of action of patient welfare that was unique to nursing. The task was to clarify nursing's social mission from the perspective of a theoretically sound view of the client.

The JBSM offers useful guidelines for nursing practice. Used in conjunction with the nursing process, it has provided a useful conceptual map to plan patient care.



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Johnson's Behavioral System Model in Nursing Practice .....

Other studies have also documented the utility of the JBSM for nursing practice. Holaday (1980) used the model to assess health status and to develop nursing interventions for children undergoing surgical procedures.

Wang and Palmer (2010) used the model to conduct an analysis of the concept of women's toileting behavior related to urinary elimination. Dee, van Servellen, and Brecht (1998) found that the JBSM could be used to derive meaningful conclusions about the impact of managed care on nursing care problems, and Coward and Wilkie (2000) demonstrated that the JBSM could be used to plan pain management for cancer patients.

Colling, Owen, McCreedy, and colleagues (2003) planned a continence program for the frail elderly; Brinkley, Ricker, and Toumey (2007) used the model to examine esthetic knowing with a hospitalized obese patient.



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Overview of Johnson's Behavioral System Model

Johnson's model for nursing presents a view of the client as a living open system. The client is seen as a collection of behavioral subsystems that interrelate to form a behavioral system. Therefore, the behavior is the system, not the individual. This behavioral system is characterized by repetitive, regular, predictable, and goal-directed behaviors that always strive toward balance (Johnson, 1968).

Johnson (1968) proposed that the nursing client is a behavioral system with behaviors of interest to nursing and is organized into seven subsystems of behavior: achievement, affiliative, aggressive, dependence, eliminative, ingestive, and sexual. Nurses using the model believed that an additional area of behavior needed to be addressed.



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They added an eighth subsystem, restorative. Each subsystem has its own structure and function. Each subsystem comprises a goal based on a *universal drive, set, choice,* and *action.* Each of these four factors contributes to the observable activity of a person.

The basis for the goal is a ***universal drive,*** the existence of which is supported by existing theory or research. The goal of each subsystem is the same for all people when stated in general terms; however, variations among individuals occur and are based on the value placed on the goal and drive strength.

The second structural component is ***set,*** which is a tendency to act in a certain way in a given situation. Once they are developed, sets are relatively stable. Set formation is influenced by such societal norms and variables as culture, family, values, perception, and perseverative sets.



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Choice refers to the alternate behaviors the person considers in any given situation. A person's range of options may be broad or narrow. Options are influenced by such variables as age, gender, culture, and socioeconomic status.

The ***action*** is the observable behavior of the person. The actual behavior is restricted by the person's size and abilities. Here them concern is the efficiency and effectiveness of the behavior in goal attainment.

- The goal of the subsystem is a part of the structure. It is not entirely separate from its function.
- The subsystems maintain behavioral system balance as long as both the internal and the external environments are orderly, organized, and predictable and each of the subsystems' goals is met. Behavioral system imbalance arises when structure, function, or functional regimen is disturbed.



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Johnson's Behavioral System Model in Nursing Practice .....

- The JBSM differentiates four diagnostic classifications to delineate these disturbances: insufficiency, discrepancy, incompatibility, and dominance.

- Nursing has the goal of maintaining, restoring, or attaining a balance or stability in the behavioral system or in the system as a whole. Nursing acts as an "external regulatory force" to modify or change the structure or to provide ways in which subsystems fulfill the structure's functional requirement.

- Interventions directed toward restoring behavioral system balance are directed toward repairing damaged structural units, with the nurse temporarily imposing regulatory and control measures or helping the client develop or enhance his or her supplies of essential functional requirements.



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Critical Thinking in Nursing Practice with Johnson's Model

- Making wise choices about nursing care requires the ability to think critically—that is, to analyze the available information, make inferences, draw logical conclusions, and critically evaluate all relevant elements, as well as the possible consequences of each nursing decision. The JBSM provides information in a way that permits problem solving and care planning (Table 5-2).

- The focus of the process is to obtain knowledge of the client through interviews and observations of the patient and family to evaluate the present behavior in terms of past patterns, to determine the effect of the present illness or perceived health threat and/or hospitalization on behavioral patterns, and to establish the maximum possible level of health toward which an individual can strive.



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Table 5-2 The Johnson Model and the Nursing Process

Framework Elements	Nursing Thought
BEHAVIORAL ASSESSMENT	
Eight subsystems:	
1. Achievement	Do I understand the patient's perceptions?
2. Affiliative	How complete are my data collections?
3. Aggressive/protective	Could I have missed something?
4. Dependency	How do I know I have the facts right?
5. Eliminative	What data might need to be verified?
6. Ingestive	How do these data compare with previously collected data?
7. Restorative	How do my client's data compare with accepted standards/
8. Sexual	behaviors for someone of this age, culture, and disease process?
ENVIRONMENTAL ASSESSMENT	
Internal:	
Biological Pathological Psychological Developmental Level of wellness	What general and specific factors supply the functional requirements for subsystems?
External:	
Cultural Ecological Familial Sociological	

Table 5-2 The Johnson Model and the Nursing Process—cont'd

Framework Elements	Nursing Thought
DIAGNOSTIC ANALYSIS	
Behavior subsystem (e.g., achievement) Structural unit: Goal, set, choice, action	What subsystem(s) is (are) involved? What structural unit(s) is (are) involved? Is behavior succeeding or failing to achieve the goal? Is there a clear relationship between the stated goal of the person, set, and chosen behavior? Does the set of the person result in misperception of the information? Are the choices appropriate? Is the sequence of action orderly and purposeful?
Sustenal imperatives: Variables from the environment (e.g., familial)	Which of the sustenal imperatives is causing or influencing the behavior(s)? What regulating and control mechanisms are present? What are the quality and quantity of sustenal imperatives?
Diagnostic label:	***Questions to guide determination of diagnosis:***
Insufficiency Discrepancy Incompatibility	What is the meaning of _____? What are the implications of _____? What do we already know about _____?
Dominance	How does _____ affect _____? Explain why _____. Explain how _____. Why is _____ important? What do you think causes _____? Why? What are the relationships between _____and _____?
PLANNING AND INTERVENTION	
Mutual goal setting Identify focus of intervention Identify mode of intervention Identify technique	*Questions to guide development of intervention:* What would happen if _____? What are some possible interventions for the diagnosis of _____? What are possible unintended consequences of the intervention?
EVALUATION	
Establish long-term goals Establish short-term goals Develop behavioral objectives to measure progress toward goals	Are goals socially, culturally, and biologically appropriate? Are the goals reasonable? Are the goals client-centered? Are the objectives measurable and theory-based?

Johnson's Behavioral System Model in Nursing Practice .....

- Concept mapping is also an effective strategy to combine with use of a nursing theory such as the JBSM (Daley, 1996).
- A concept map is a graphic or pictorial arrangement of key concepts that address specific subject matter such as behavioral system imbalance.
- Concept maps are organized with the most important or central concept at the top of or in the center of the paper.
- Related information or issues are to the side or underneath the main topic.
- There is no right or wrong way to design a concept map. It may be a flowchart or a diagram or even shaped like a heart if the issue were congenital heart disease.
- The purpose of the concept map is to develop critical thinking skills and problem-solving abilities.



2-King's Conceptual System and Theory of Goal Attainment in Nursing Practice

Dr. Hala Aiash

2020-2021

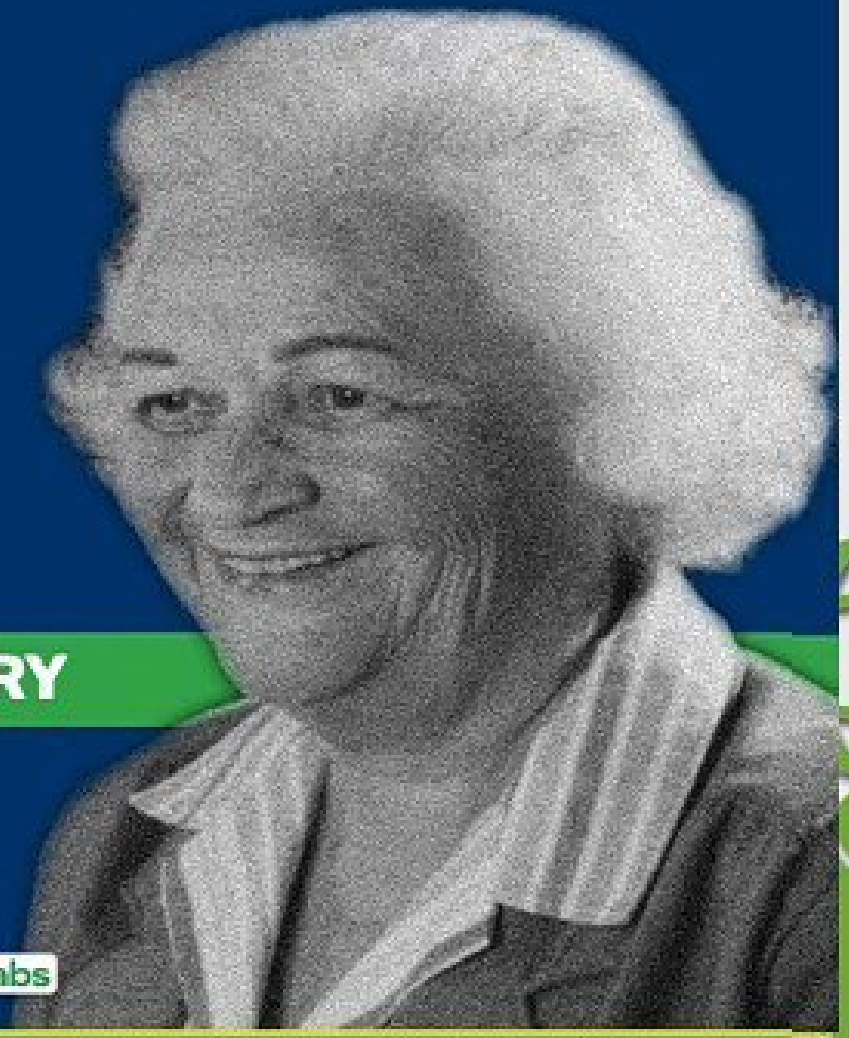


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NURSING THEORIES GUIDE

IMOGENE KING

GOAL ATTAINMENT THEORY



Nurseslabs

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Nurses are the key persons in the health care system who identify the goals and the means to help individuals and families attain goals.

(King, 1995b, p. 24)

History and Background

In 1971, King proposed a conceptual system for nursing around four concepts she considered universal to the discipline of nursing: social systems, health, perception, and interpersonal relationships. These areas were identified from the synthesis and reformulation of concepts using inductive and deductive reasoning, critical thinking, and extensive review of nursing and literature from other health-related disciplines.



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- King expanded the conceptual system during the 1970s by further explicating the nature of persons and environment, strengthening the general systems orientation, and expanding the concepts.
- Concepts in the personal system were perception, self, growth and development, body image, time, and space. Concepts in the interpersonal system were human interaction, communication, transactions, role, and stress. Concepts in the social system were organization, authority, power, status, and decision making.
- King explicated the philosophical basis and enduring nature of the conceptual system and theory for nursing with emphasis on the twenty-first century and the world as community.
- Overall, King's conceptual system and Theory of Goal Attainment represent significant theoretical structures for theory development and theory-based practice for nursing



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Overview of King's Conceptual System and Theory of Goal Attainment

- King's conceptual system is based on the assumption that human beings are the focus of nursing.

- The linkage between interactions and health is behavior, or human acts. Nurses must have the knowledge and skill to observe and interpret behavior and intervene in the behavioral realm to assist individuals and groups cope with health, illness, and crisis (King, 1981).
- According to King (1981, 1988, 1991), concepts are critical because they provide knowledge that is applicable to practice. Systems and concepts within King's conceptual system and Theory of Goal Attainment are described and defined in the following :



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Personal Systems

Each individual is an open, total, unique system in constant interaction with the environment. Interactions between and among personal systems are the focus of King's conceptual system. The following concepts provide foundational knowledge that contributes to understanding individuals as personal systems:

- *Perception:* "A process of organizing, interpreting, and transforming information from sense data and memory"
- *Self:* "Self is what I think of me and what I am capable of being and doing. Self is subjective in that it is what I think I should be or would like to be". Self is a dynamic, action-oriented open system.
- *Growth and development:* "The processes that take place in an individual's life that help the individual move from potential capacity for achievement to self-actualization"



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- *Body image:* "An individual's perceptions of his/her own body, others' reactions to his/her appearance which results from others' reactions to self"
- *Learning:* "A process of sensory perception, conceptualization, and critical thinking involving multiple experiences in which changes in concepts, skills, symbols, habits, and values can be evaluated in observable behaviors and inferred from behavioral manifestation".
- *Time:* "Duration between the occurrence of one event and occurrence of another event"
- *Personal space:* "Existing in all directions and is the same everywhere"
- *Coping:* King (1981) used the term *coping* in her discussion of the concept of stress in the interpersonal system and in later discussions of the Theory of Goal Attainment without explicit definition.



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Interpersonal Systems

Interpersonal systems are formed by the interactions of two or more individuals Concepts critical to understanding interactions between individuals are defined as follows:

- *Communication:* "Information processing, a change of information from one state to another"
- *Interaction:* "The process of interactions between two or more individuals represents a sequence of verbal and nonverbal behaviors that are goal-directed"
- *Role:* "Set of behaviors expected when occupying a position in a social system"



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- *Stress:* "Dynamic state whereby a human being interacts with the environment to maintain balance for growth, development, and performance which involves an exchange of energy and information between the person and the environment for regulation and control of stressors"

- *Stressors:* Events that produce stress .

- *Transaction:* "Observable behaviors of human beings interacting with their environment"



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Social Systems

A social system is defined as "an organized boundary system of social roles, behaviors, and practices developed to maintain values and the mechanisms to regulate the practice and rules"

Concepts that are useful to understand interactions within social systems and between social and personal systems are defined as follows:

- *Organization:* "A system whose continuous activities are conducted to achieve goals"

- *Authority:* "Transactional process characterized by active, reciprocal relations in which members' values, backgrounds, and perceptions play a role in defining, validating, and accepting the [directions] of individuals within an organization"



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King's Conceptual System and Theory of Goal Attainment in Nursing Practice .....

- Power: "The capacity or ability of a group to achieve goals"
- Status: "The position of an individual in a group or a group in relation to other groups in an organization" .

- Decision making: "Dynamic and systematic process by which a goal directed choice of perceived alternatives is made, and acted upon, by individuals or groups to answer a question and attain a goal"

- King's conceptual system provides both structure and function for nursing. Clearly stated assumptions about persons, environment, health, nursing, and systems provide a conceptual orientation of holism and dynamic interaction, specify health as the goal of nursing, and actively include the patient (individual, family, or community) in decisions about setting goals and the behavior necessary to achieve health goals.



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Theory of Goal Attainment

- The Theory of Goal Attainment addresses nursing as a process of human interaction.
- The nurse and patient form an interpersonal system in which each affects the other and in which both are affected by situational factors in the environment. Drawn from both the personal and interpersonal system concepts, the Theory of Goal Attainment comprises the concepts of perception, communication, interaction, transaction, self, role, growth and development, stress/stressors, coping, time, and personal space.
- King identified that perception, communication, and interaction are essential elements in transaction. When transactions are made, goals are usually attained. The human interaction and conceptual focus dimensions of the theory guide the nursing process dimension (Figure 5-1)



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King's Conceptual System and Theory of Goal Attainment in Nursing Practice .....

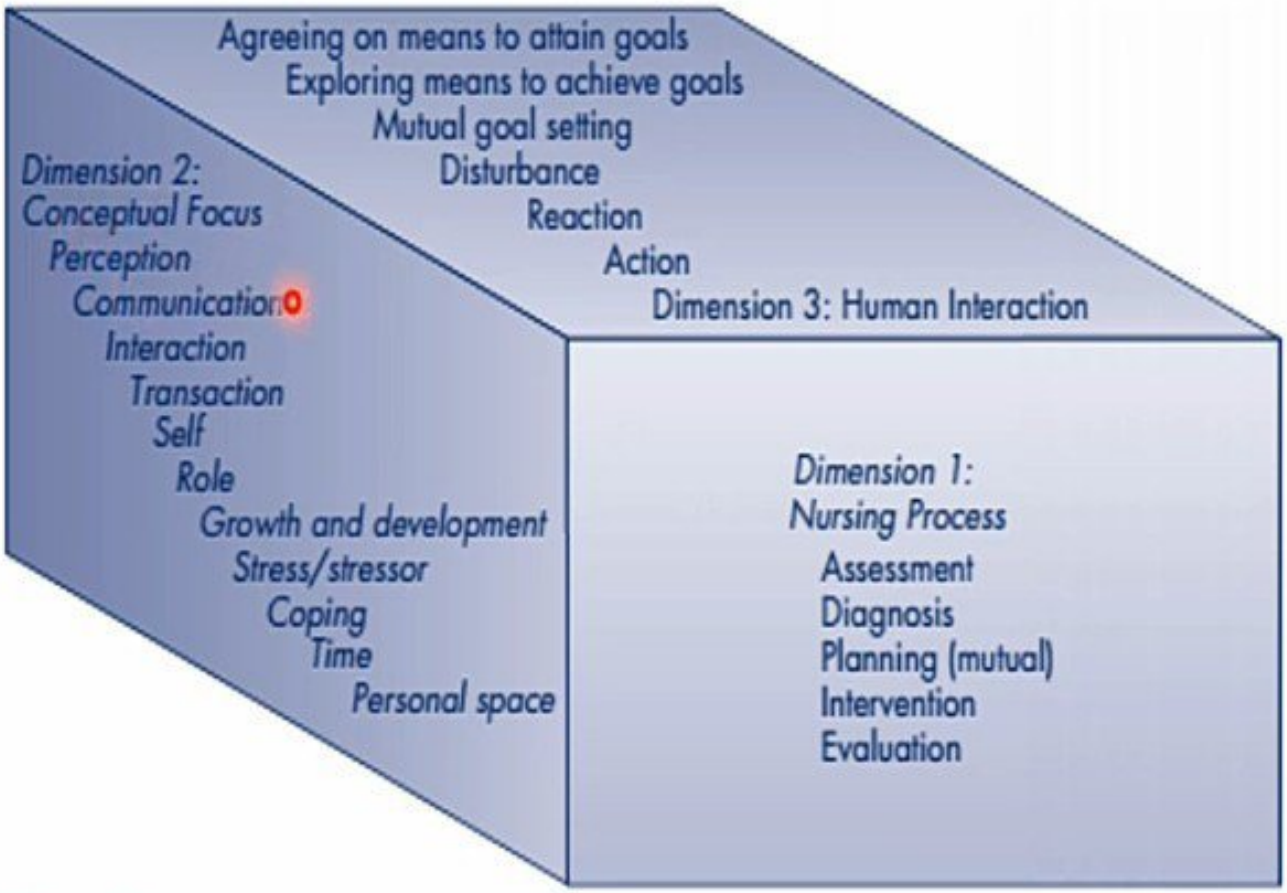


Figure 5-1

Three-dimensional nursing process based on King's Theory of Goal Attainment. (Modified from Alligood, M. R. [1995]. Theory of goal attainment: Application to adult orthopedic nursing. In M. A. Frey & C. L. Sieloff [Eds.], Advancing King's systems framework and theory of nursing [p. 212]. Thousand Oaks, CA: Sage.)

Critical Thinking in Nursing Practice with King's Conceptual System

- The development and use of critical thinking in nursing has received considerable attention both in nursing education and in practice over the past two decades.
- Critical thinking has always been an integral component in King's perspective of nursing. King explicitly linked critical thinking to the mental acts of judgment that are implicit in perception, communication, and interactions leading to transaction and the concept of decision making.
- Critical thinking is illustrated by the use of terms such as *analyze, synthesize, verify,* and *interpret.* King explicitly linked critical thinking to the mental acts of judgment that are implicit in perception, communication, and interactions leading to transaction and the concept of decision making.



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- The following discussion illustrates critical thinking questions based on concepts within King's systems framework that are essential in carrying out activities of assessing, planning, implementing, and evaluating.

- At the first step of King's process of nursing, the nurse meets the patient and communicates and interacts with him or her. Assessment is conducted by gathering data about the patient based on relevant concepts. The end result of these critical thinking activities is a comprehensive patient assessment tailored to the patient and his or her situations .

- The next step of King's process of nursing is identifying goals and planning to achieve those goals. This step is congruent with planning in the traditional nursing process.



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- The third step in King's process of nursing results in transactions being made. Transactions occur as a result of perceiving the other person(s) and the situation, making judgments about those perceptions, and taking some action in response. Reaction to action(s) leads to interaction between the nurse and patient, which leads to transactions that reflect a shared view and commitment.

  This step reflects implementation in the traditional nursing process.

- The fourth step in King's process of nursing is goal attainment or failure to attain the goal. This exercise in critical thinking is comparable to the evaluation step in the traditional nursing process.



In summary, the traditional nursing process is a system of interrelated actions, the methods by which nursing is practiced. The critical thinking process reflects highly developed thinking skills essential for nursing practice in the twenty-first century. The transaction process (goal attainment) requires knowledge of concepts from King's conceptual system as well as knowledge of those professional interactions described in the Theory of Goal Attainment. King (1999) compared the nursing process, the critical thinking process, and the transaction process to illustrate the application of her theory when addressing ethical issues in contemporary nursing practice.



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Orem's Self-Care Deficit Theory in Nursing Practice

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2020-2021



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NURSING THEORIES GUIDE

DOROTHEA OREM

SELF-CARE DEFICIT THEORY



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Nurses work in life situations with others to bring about conditions that are beneficial to persons nursed. Nursing demands the exercise of both the speculative and practical intelligence of nurses. In nursing practice situations, nurses must have accurate information and be knowing about existent conditions and circumstances of patients and about emerging changes in them. This knowledge is the concrete base for nurses' development of creative practical insights about what can be done to bring about beneficial relationships or conditions that do not presently exist. Asking and answering the questions "what is?" and "what can be?" are nurses' points of departure in nursing practice situations.

(Orem, 1995, p. 155)



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Orem's Self-Care Deficit Theory in Nursing Practice .....

History and Background

- The Self-Care Deficit Nursing Theory (SCDNT) is one of the nursing theories most commonly used in practice (Im & Chang, 2012). Orem's dedication to the concept of self-care resulted in a nursing theory appropriate for present and future health care scenes.

- Orem's purpose was to define the following:
  - Nursing's concern—"man's need for self-care action and the provision and management of it on a continuous basis in order to sustain life and health, recover from disease or injury, and cope with their effects" (Orem, 1959, p. 3)
  - Nursing's goal—"overcoming human limitations" (Orem, 1959, p. 4)



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Orem's Self-Care Deficit Theory in Nursing Practice .....

- Orem's model supports nursing through the following three central theories:

1. Nursing is required because of the inability to perform self-care as the result of limitations (Theory of Self-Care Deficit).
2. Maturing or mature adults deliberately learn and perform actions to direct their survival, quality of life, and well-being (Theory of Self-Care).
3. The product of nursing is nursing system(s) by which nurses use the nursing process to help individuals meet their self-care requisites and build their self-care or dependent-care capabilities (Theory of Nursing Systems).



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Orem's Self-Care Deficit Theory in Nursing Practice .....

- Early use of the theory in practice began with the work of the Nursing Development Conference Group (NDCG) (1973). The group initiated their adventure into theory-based practice by integrating the developing concepts of the model into their clinical teaching. As the conceptualizations evolved, they were incorporated into nursing care.

- Gradually, SCDNT development in the practice arena began to filter into a variety of practice settings. In the 1980s the influence of the SCDNT on practice was accelerated. This was a time when the theory was being explicated for use with specific nursing situations and in varying types of practice settings. The literature of the 1990s shows SCDNT-guided practice in a variety of settings and situations.



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Overview of Orem's Self-Care Deficit Nursing Theory

Nursing practice oriented by the SCDNT represents a caring approach that uses experiential and specialized knowledge (science) to design and produce nursing care (art). The body of knowledge that guides the art and science incorporates empirical and antecedent knowledge .

Orem (1995) identified eight fields of knowledge essential for understanding nursing practice. Seven of those emanate from previously developed fields of knowledge found in the sciences and other disciplines, including sociology, profession/occupation, jurisprudence, history, ethics, economics, and administration. The eighth, nursing science is knowledge about nursing practice created by nurses through scientific investigations that yield understanding of the field of nursing and provide foundations for nursing practice.



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Practice knowledge is systematized, validated, and conducive to dynamic processes. Its dynamic quality leads the user to acceptance and owning of the theory (Orem, 1988). Allison (1988) noted the dynamic quality of the SCDNT and commented that the theory always keeps the nurse in an action mode. Orem (1988) emphasized that today's nurses must be scholars within the developing theory. In doing so, nurses are committed to an awareness of the relationship between what they know and what they do. From this awareness comes a healthy sense of professionalism



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Critical Thinking in Nursing Practice with Orem's Theory

SCDNT-based critical thinking emanates from four structured cognitive operations: diagnostic, prescriptive, regulatory, and control. Each operation fulfills a distinct phase in the use of the theory. Sequencing of the phases may vary throughout the process in order to reassess and continue to prescribe and regulate the nursing system for the best interest of self-care.

The operations are intended to be collaborative and to provide the self care agent or dependent-care agent input into the decision making.



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Diagnostic Operations

- The first phase, diagnostic operations , begins with establishing the nurse-client relationship and proceeds to contracting to work toward identifying and discussing current and potential therapeutic self-care demands.
- An analysis of the assessment data results in a diagnosis concerning the type of self-care demands. From these data, inferences about the adequacy and potential of self-care agency are made, validated, and treated as diagnostic of self-care agency.
- Finally, self-care deficits are diagnosed by reflecting on the adequacy of agency to meet specific requisites. In instances in which self-care agency is inadequate, a self-care deficit is stated.



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➢ Prescriptive Operations

- In the prescriptive phase, ideal therapeutic self-care requisites for each self-care requisite are determined by reviewing possible helping methods, considering related basic conditioning factors, and identifying the most appropriate helping methods.
- Actions required for the therapeutic self-care demands are discussed with the client and are designed for maximal efficiency and compatibility.
- Priority is given to those therapeutic self-care demands that are the most essential to physiological processes. Client and nurse expectations are formalized and recognized as supportive of continued development of self-care agency.



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➢ Regulatory Operations

- ♣ The prescriptions that evolve are used in the regulatory phase to design, plan, and produce the regulatory nursing system. Factors entering decisions about design include basic conditioning factors, effective regulation of health and developmental state, timing, assignment of actions, and degree of cooperation.

- ♣ Further planning specifies conditions for the regulatory operations such as frequency, equipment and supplies, and personnel needed. Throughout the production of regulatory care, there is emphasis on development of self-care agency by using helping methods that encourage learning, increase feelings of well-being, and stimulate interest in self-care.



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Control Operations

- Evaluation occurs in the control phase. The effectiveness of regulatory operations and client outcome is estimated. Regulatory operations are evaluated for correctness and appropriateness. Client outcome is appraised for regulation of functioning, developmental change, and adjustments to varying levels of self-care ability.



Neuman Systems Model in Nursing Practice

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2020-2021



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NURSING THEORIES GUIDE

BETTY NEUMAN

NEUMAN SYSTEMS MODEL



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The philosophic base of the Neuman Systems Model encompasses wholism, a wellness orientation, client perception and motivation, and a dynamic systems perspective of energy and variable interaction with the environment to mitigate possible harm from internal and external stressors, while caregivers and clients form a partnership relationship to negotiate desired outcome goals for optimal health retention, restoration and maintenance.

(Neuman, 2011, p. 12)



History and Background

- The Neuman Systems Model was first developed in 1970 to assist graduate students to consider patient needs in wholistic terms (Neuman, 1974, 2011).
- Neuman Systems Model is a synthesis of systems thinking and wholism that provides a comprehensive systems approach for wellness-focused nursing care.
- Neuman considers "client" to be an individual, a group, a family, or a community system. Each client is viewed with five variables that interact synergistically in relation to each other and reciprocally with the internal, external, and created environments in which the client exists.



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Neuman Systems Model in Nursing Practice .....

- The five client variables essential to the Neuman model are physiological, psychological, developmental, sociocultural, and spiritual. Intrapersonal, interpersonal, or extrapersonal environmental stressors can affect potential or actual reactions within the client system.
- Neuman uses the term *wholism* to reference biological and philosophical concepts "implying relationships and processes arising from wholeness, dynamic freedom, and creativity in adjusting to stressors in the internal and external environments" (Neuman, 2011, p.) The Neuman Systems Model incorporates the structure and process components of open systems models.



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Overview of Neuman Systems Model

Application of the Neuman Systems Model in nursing praxis can occur in any setting. In each nursing situation, the nurse completes a wholistic assessment of actual or potential stressors, client variables, and boundary impact. The nurse determines the client's perspective before the analysis and synthesis of the objective and subject data collection. In addition, client strengths, weaknesses, and resources are considered.

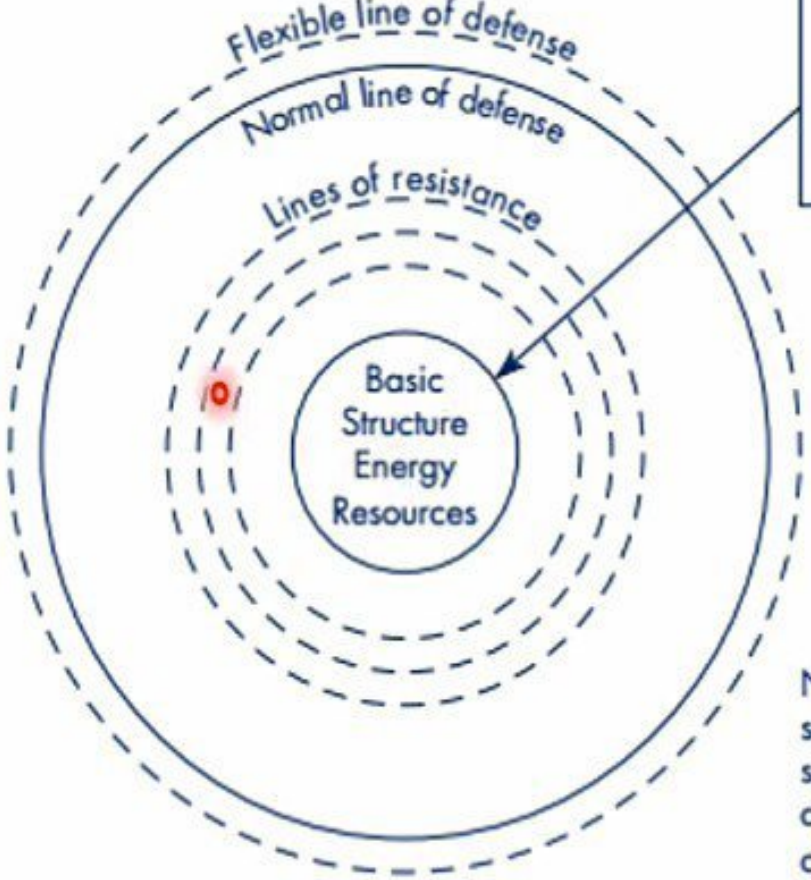
The aim of the Neuman model "is to set forth a structure that depicts the parts and subparts and their interrelationship for the whole of the client as a complete system" (Neuman, 2011, p. 12).

As depicted in Figure 5-2, and beginning from the center of the figure, the Neuman model identifies a basic structure of energy resources, variables, system boundaries, and the environment as the core subparts of the system.



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Neuman Systems Model in Nursing Practice ……..



Basic structure
Basic factors common to all organisms, e.g.,
- Normal temperature range
- Genetic structure
- Response pattern
- Organ strength or weakness
- Ego structure
- Known commonalities

NOTE: Physiological, psychological, sociocultural, developmental, and spiritual variables occur and are considered simultaneously in each client concentric circle.

Figure 5-2

The Neuman Systems Model. (From Neuman, B., & Fawcett, J. [2011]. *The Neuman Systems Model* [5th ed., p. 15]. Boston: Pearson.)

Neuman Systems Model in Nursing Practice .....

- At the core of the diagram, energy resources are noted. A constant energy exchange occurs between the client system and environment. The client maintains and augments system stability by using energy, regarded as a positive force available to the system. As such, stability is not static but adaptive and developmental in nature because the client system is considered an open system in a state of constant change.

- A series of protective rings encircle the center structure and protect the system from environmental stressors. Each system component is intersected by five variables (physiological, psychological, sociocultural, developmental, and spiritual). These five variables interact synergistically and wholistically within all parts of the client system.



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Neuman Systems Model in Nursing Practice .....

- Nursing actions necessary for the client system to attain restabilization (reconstitution) through energy conservation and the use of internal and external resources are considered secondary preventions. These interventions protect the basic structure of the system. The nurse implements secondary prevention actions whenever stressor reactions occur and symptoms are present.

- After a therapeutic modality and reconstitution, the maintenance of client system stability is achieved by tertiary prevention-as-interventions (Neuman, 2011). Nursing actions such as education and reinforcement about nutrition, exercise, and medications that can maintain the reconstitution of the client with hypertension are examples of tertiary prevention.



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Critical Thinking in Nursing Practice with Neuman's Model

- The Neuman Systems Model provides a structure for critical thinking in several ways.

- This systems-based model allows for reconceptualization when clinical situations undergo rapid change. In such rapid change situations, the Neuman Systems Model allows for the identification of interrelationships between identified systems, parts, subparts, and the environment, leading to consequent and rational nursing actions.

- Neuman Systems Model nursing process categories of nursing diagnoses, nursing goals, and nursing outcomes create a format for purposeful critical thinking and problem solving that translates to action.



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Neuman Systems Model in Nursing Practice .....

- Comprehensive assessment based on the Neuman Systems Model requires a conceptualization of each model component (client perspective, variables, basic structure, environmental stressors, and boundaries) by the nurse and is essential to determine current client status accurately. The nurse-client relationship begins at first contact and is conceived as an ongoing partnership between the nurse and client.
- Earlier Neuman guidelines for use of the model in clinical practice stipulated that the diagnostic taxonomy reflected the model in the following categories: (1) client system (individual, family, group, community); (2) response level (primary, secondary, tertiary); (3) subsystem response (fve variables); (4) stressor source (intrasystem, intersystem, extrasystem); and (5) stressor type (five variables) .It seems clear that the synthetic use of the Neuman Systems Model and nursing process facilitates enhanced critical thinking by the nurse.



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3- Levine's Conservation Model in Nursing Practice

Dr. Hala Aiash

2020-2021



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NURSING THEORIES GUIDE

MYRA ESTRIN LEVINE

CONSERVATION MODEL



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Levine's Conservation Model in Nursing Practice

Nursing is a profession as well as an academic discipline, always practiced and studied in concert with all of the disciplines that together form the health sciences.... Scientific knowledge from many contributing disciplines is, in fact, connected to nursing, as an adjunct to the knowledge that nursing claims for its own. (Levine, 1988, p. 17).

History and Background

- The conservation model was originally an organizing framework for teaching undergraduate nursing students
- Levine taught the skill of nursing and the rationale for the actions. She demonstrated high regard for the contribution of the adjunctive sciences to a theoretical basis of nursing in a clear voice for discipline development and attention to the rhetoric of nursing theory.



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Levine's Conservation Model in Nursing Practice …….

- The universality of the conservation model is supported by its use with a variety of patients of varied ages in a wide range of settings, including critical care , acute care , and long-term care. It is also used with neonates, infants young children….
- The model is used for quantitative and qualitative research addressing practice issues.
- Chang (2007) studied swaddling guided by Levine's model and found swaddling conserved energy because the heart rates of swaddled infants increased less than those of the control group. Watanabe and Nojima (2005) proposed a middle range theory describing the "calm delivery" and conservation of social integrity relationship based on key indicators of parental relationships, person-environment relationship, family function, dyadic relationship between parents, and dyadic relationship between the generations



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Overview of Levine's Conservation Model

- According to Levine (1973a), "Nursing is human interaction" . "The nurse enters into a partnership of human experience where sharing moments in time some trivial, some dramatic leaves its mark forever on each patient"
- Three major concepts form the basis of the model and its assumptions: (1) conservation, (2) adaptation, and (3) wholeness. Conservation is a natural law fundamental to many sciences.
- Conservation is the keeping together of the life system. *To keep together* means to maintain a proper balance between active nursing interventions and patient participation, in which the patient participates within the safe limits of his or her ability.
- Conservation is about achieving balance between energy supply and demand within the unique biological realities of the individual.



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Levine's Conservation Model in Nursing Practice .....

- Levine (1988) referred to the metaparadigm concepts of person, environment, health, and nursing as *commonplaces of the discipline* because they are necessary for any description of nursing. Persons are holistic beings who are sentient, thinking, future-oriented, and aware of their past.
- Persons are in constant interaction with the environment, responding to change in an orderly, sequential pattern, adapting to forces that shape and reshape their essence. According to Levine (1973a), the person can be defined as an individual, a group (family), or a community of groups and individuals (Pond, 1991).
- The environment completes the wholeness of the person. Each individual has his or her own internal and external environments.
- The internal environment includes physiological and pathophysiological aspects of the patient that are challenged by changes in the external environment. External environmental factors impinge on and challenge the individual



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Levine's Conservation Model in Nursing Practice .....

- The goal of nursing is to promote adaptation and maintain wholeness. This goal is accomplished through the conservation of energy and structural, personal, and social integrity.
- Conservation of energy is integral to individual ranges of adaptive responses.
- Conservation of structural integrity depends on the defense system that supports repair and healing in response to challenges from internal and external environments.
- Conservation of personal integrity recognizes individual wholeness in response to environment as the individual strives for recognition, respect, self-awareness, humanness, holiness, independence, freedom, selfhood, and self-determination.
- Conservation of social integrity recognizes individual functioning in a society that helps establish the boundaries of the self. Social integrity is created with family and friends, workplace and school, religion, personal choices, and cultural and ethnic heritage



Theories for Practice from the Model

The model provides a basis for the following four theories for practice (Mefford, 2004):

1. The Theory of Conservation
2. The Theory of Therapeutic Intention
3. The Theory of Redundancy
4. The Theory of Health Promotion for Preterm Infants (Mefford, 2000)

Critical Thinking in Nursing Practice with Levine's Model

Levine (1973a,b) proposes that nurses use their scientific and creative abilities to provide nursing care to patients using a process incorporating ability to think critically.



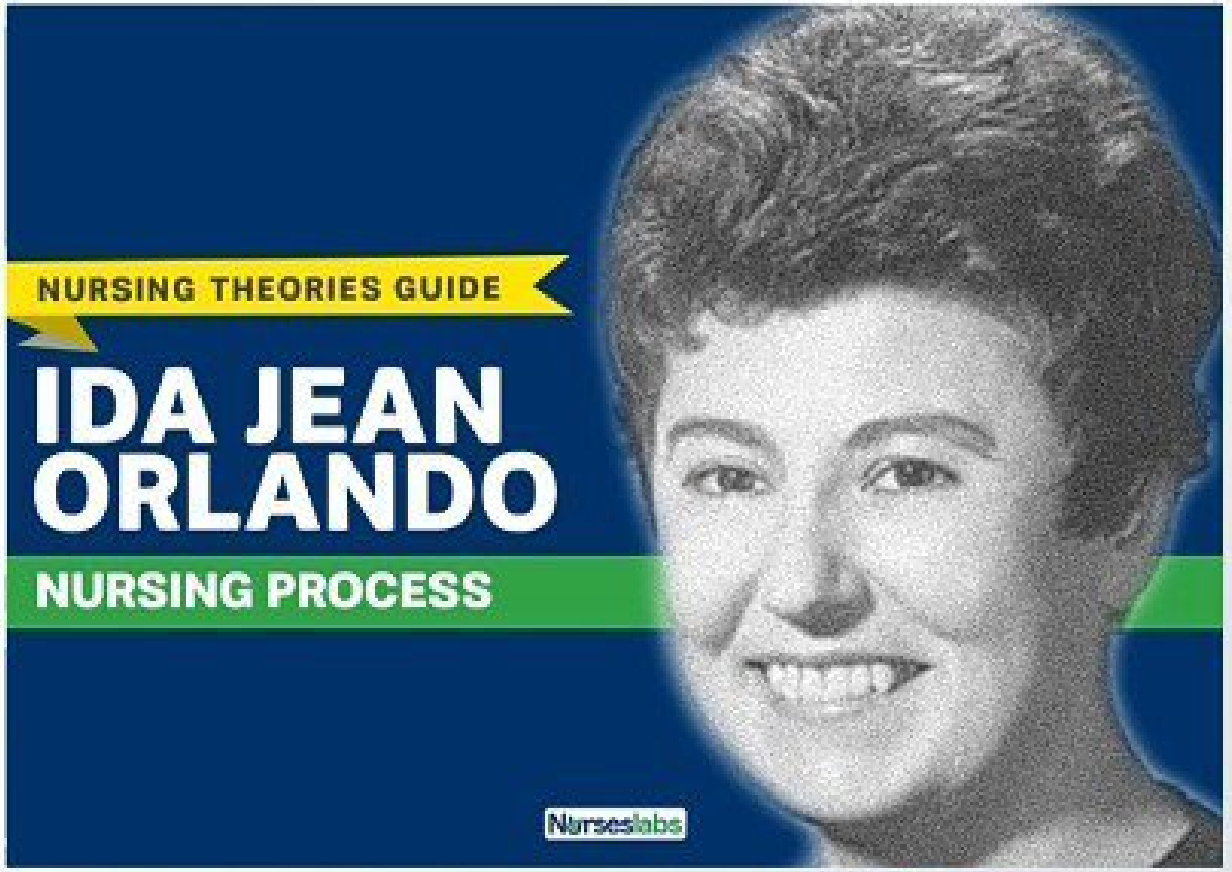
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Nursing Theory Utilization & Application

Assist. Prof. Dr. Hala Aiash

2020-2021

Orlando's Nursing Process Theory in Nursing Practice



Orlando's Nursing Process Theory in Nursing Practice .....

History and Background

- In the 1950s, governmental grants became available for studies integrating mental health concepts into nursing curricula. Yale University received such a National Institute of Mental Health grant and Ida Orlando became project investigator. Orlando was the first to develop theory inductively in an empirical study of nursing practice. After examining 2000 records, Orlando was able to categorize them as "good" and "bad" nursing.

Orlando's Nursing Process Theory in Nursing Practice .....

- Orlando believed that if they all agreed, then the record's anecdotal content contained "what made good and bad nursing happen" .
- According to Orlando, "In the records judged as good the nurse's focus was on the patient's immediate verbal and nonverbal behavior from the beginning through the end of the contact; whereas in those judged bad, the nurse's focus was on a prescribed activity or something that had nothing to do with the patient's behavior"

Orlando's Nursing Process Theory in Nursing Practice .....

- Orlando's theory is appealing because it clearly describes what nurses consider good nursing and she proposed its value for the twenty-first century Orlando's theory has been used for and by both clinical and administrative practice in two acute-care hospitals ,in operating rooms ,in a psychiatric hospital ,in nursing management ,and in a longterm care facility .Orlando's work has been translated into six languages.

Overview of Orlando's Nursing Process Theory

Orlando's theory is a reflective practice theory that is based on discovering and resolving problematic situations. If the problem is not discovered, it cannot be solved. The centrality of the patient is ever-present when using Orlando's theory. Orlando described her theory as a *nursing process theory* rather than *effective nursing care* theory .

Orlando's Nursing Process Theory in Nursing Practice .....

Framework of the Theory

As a reflective practice theory, Orlando's theory contains concepts that are interrelated but are described separately. These five interrelated concepts are addressed within the problematic framework derived from Schmieding's (1983, 1987) analysis of Orlando's theory using the writings of John Dewey . The five concepts are the following:

1. Professional nursing function—organizing principle
2. Patient's presenting behavior—problematic situation
3. Immediate reaction—internal response
4. Deliberative nursing process—reflective inquiry
5. Improvement—resolution

Critical Thinking in Nursing Practice with Orlando's Theory

- Consequently, critical thinking requires a reflective process that is patient-centered (Daly, 1998) and also enhances nurses' functioning in the complex health environment.
- In contrast to the nursing process, critical thinking is characterized as a "unique, cognitive thought process that is grounded in reflection". Kataoka-Yahiro and Saylor (1994) stress that attitudes toward critical thinking, knowledge fundamental to nursing practice, experience that leads to understanding complex situations, and cognitive competencies are needed for nursing judgment.

Orlando's Nursing Process Theory in Nursing Practice .....

- In Orlando's theory, ascertaining the patient's immediate need for help emphasizes the importance Orlando gives to the first step of the critical thinking process.
- If the problem is inaccurately identified, the development of goals, strategies, and other components of the nursing process will not be effective because they are based on a faulty foundation.
- In all aspects of the nursing process, Orlando's theory places high priority on patient involvement. In essence, the nurse and patient are partners in the process.
- Orlando's theory also emphasizes that patient improvement is determined by positive changes in both the verbal and the nonverbal behavior of the patient, which from Dewey's perspective is the basis of evidence.

Nursing Theory Utilization & Application

Assist. Prof. Dr. Hala Aiash

2020-2021

Helen Erickson



Modeling and Role-Modeling Theory in Nursing Practice

Modeling and Role-Modeling is based on the philosophy that all humans have the desire to live healthy, happy lives, to find meaning and purpose in their lives, and to become the most that they can be. This holds true across the lifespan. When we use strategies that focus on the strengths of our clients, help them become more fully alive (even as they approach physical death), and to live their lives to the fullest, then we are truly helping them grow, heal, and transcend. We help them discover the essence of their being, to find or reclaim their soul.

(Erickson, 2001, p. 309).

Modeling and Role-Modeling Theory in Nursing Practice .....

History and Background

The Modeling and Role-Modeling (MRM) paradigm, conceived by Helen Erickson in the late 1950s,was an outcome of her exposure to the work of her father-in-law, Milton H. Erickson, coupled with her own experiences as a professional nurse.

During the late 1940s and the 1950s, M. Erickson, MD, developed an international reputation for his unorthodox methods, views on human nature, and clinical results. H. Erickson states that she repeatedly asked him to tell her "what to do and how to do it"







Modeling and Role-Modeling Theory in Nursing Practice .....

He hoped for protocols, treatment recommendations, and quick fixes, but instead she was told that she must model the client's world and plan strategies within that context.

M. Erickson, MD, advised that each human has a unique view of the world and needs to maintain his or her role in unique ways and that the practice of health care professionals was to help clients succeed in living quality lives and growing to the maximum of their potential.

H. Erickson's first independent research study occurred during graduate school. Under Swain's supervision, a study was designed to test the Adaptive Potential Assessment Model (APAM), which had been conceived, labeled, and articulated in the mid-1970s.

Overview of Modeling and Role-Modeling

Modeling and Role-Modeling, conceived within the context of numerous philosophical assumptions, was labeled and articulated by Erickson synthesizing several concepts from established theories. Concepts were drawn from the works of Maslow to create a new theory that described relations among needs, loss, grief, adaptation, developmental processes, growth, and well-being of the holistic person .The practice paradigm was derived from integrating philosophical assumptions with theoretical underpinnings.







Philosophical Assumptions and Constructs

Holism

Humans consist of cognitive, biophysical, social, and psychological subsystems permeated by genetic predispositions and a spiritual drive. health can be defined as a dynamic, eudemonistic sense of well-being (Erickson, et al., 2005) associated with self-fulfillment and transcendence beyond the objective reality of the moment

Affiliated -Individuation

Essential to a person's sense of well-being is the need for affiliated-individuation

(AI). AI, coined by Erickson, is defined as "the need to be dependent on support systems while simultaneously maintaining independence from these support systems. Although AI is a lifelong need for all people, it is commonly not recognized as acceptable behavior as people get older and are expected to be independent.

Modeling and Role-Modeling Theory in Nursing Practice .....

Need Satisfaction, Growth, and Development

Humans are in a continual state of change and have inherent drives that motivate behavior. These include a drive for needs satisfaction, adaptation, and growth sequential development. According to Erickson and colleagues (2005), "Growth is defined as the changes in body, mind, and spirit that occur over time" and facilitates an individual's development. Development is defined as "the holistic synthesis of the growth-produced…differentiations in a person's body, ideas, social relations, and so forth" .When individuals are given necessary information, adequate emotional support, and are empowered in making satisfactory decisions, growth and subsequent development occur and health is enhanced.

Modeling and Role-Modeling Theory in Nursing Practice .....

Internal and External Resources

According to the MRM paradigm, the nurse facilitates an interactive, interpersonal relationship with the client. During this process the nurse assists the client in identifying, developing, and mobilizing internal and external resources—resources needed to cope with life's stressors, to grow and heal. Essential to this process is the nurse's unconditional acceptance of the client. Erickson and colleagues (1983) argue that "accepted as a unique, worthwhile, important individual—with no strings attached—is imperative if the individual is to be facilitated in developing his or her own potential"

Modeling and Role-Modeling Theory in Nursing Practice .....

Modeling

In a supportive and caring environment, a nurse attempts to understand "the client's personal model of his or her world and to appreciate its value and significance for the client from the client's perspective" .The act of developing an image and understanding of the clients' worldviews from within their perspectives and framework is called *modeling.* "The way an individual communicates, thinks, feels, acts, and reacts—all of these factors comprise the individual's *model of his or her world"*







Modeling and Role-Modeling Theory in Nursing Practice .....

Role-Modeling

After the client's world has been modeled, the nurse facilitates and nurtures the individual "in attaining, maintaining, or promoting health through purposeful interventions" (Erickson, et al., 1983, p. 254). This is known as role-modeling. In role modeling the client's world, the nurse plans interventions that do the following:

- Identify mutual nurse-client goals
- Promote client strengths, control, and positive orientation
- Build trust

These interventions are aimed at helping the client "achieve an optimal state of perceived health and contentment"

Major Theoretical Linkages in Modeling and Role-Modeling

1. There is a relationship between adaptive potential and needs satisfaction.
2. There is a relationship between developmental task resolution and needs satisfaction.
3. There is a relationship between developmental task resolution and developmental residual
4. There is a relationship between developmental residual and self-care resources.
5. There is a relationship among basic needs satisfaction, object attachment, loss, grief, growth, and development.

Mercer's Becoming a Mother Theory in Nursing Practice

Dr. Hala Aiash

2020-2021

Ramona Mercer's



Mercer's Becoming a Mother Theory in Nursing Practice

Although guidelines, printed handouts, and educational materials are important, they cannot replace a nurse's dialogue with the mother that leads to identifying and understanding her concerns. Nurses are in a unique position to have long term positive effects on mothers during this transition.

(Mercer, 2006, p. 650)

History and Background

- Mercer's Theory of Maternal Role Attainment was based on her extensive research on the topic beginning in the late 1960s and early 1970s. The initial stimulus for the development of the theory came from Reva Rubin, who was Mercer's professor and mentor at the University of Pittsburgh, where Mercer earned a PhD .Rubin's (1961, 1967a,b) work in defining and understanding maternal role attainment served as a foundational theory for obstetric nursing care for many years. Rubin (1977) described maternal role attainment as a process of "binding-in" (being attached to the child) and "maternal role identity"

- Whereas Rubin focused on maternal role attainment during pregnancy and the first month after birth, Mercer expanded her concepts to include the first year following birth

Mercer's Becoming a Mother Theory in Nursing Practice .....

- In addition, Mercer considered the parents, the influence of high-risk pregnancy, and maternal illness in her theory of role attainment.
- she studied the needs and concerns of breastfeeding mothers, adolescent mothers, and mothers with postpartum illness, as well as the response of fathers to stress and complications during the childbearing process.
- Mercer introduced a framework for studying factors that affect the maternal role. The framework was more clearly defined in her book *First-Time Motherhood: Experiences from Teens to Forties.*
- Mercer (2004) explained that *becoming a mother* connotes continued growth and change in the mothering role and is more descriptive of the process. Mercer writes of motherhood as a lifelong commitment evolving over time.

Mercer's Becoming a Mother Theory in Nursing Practice .....

Overview of Mercer's Original Theory of Maternal Role Attainment

- Mercer not only relied on the previous works of Rubin but also based her research on both role and developmental theories.
- In addition, she selected study variables from an extensive review of the literature, borrowed from several disciplines, and used a variety of research tools. Her studies served as the platform for the design and development of her Theory of Maternal Role Attainment.
- Many of the assumptions, definitions, and concepts are based on Rubin's work, transition theories, and the role theories of Thorton and Nardi (1975).

Mercer's Becoming a Mother Theory in Nursing Practice .....

- Mercer's (1979) definition of maternal role attainment was based on Rubin's description of the process:

The maternal role may be considered…attained when the mother feels internal harmony with the role and its expectations. Her behavioral responses to the role's expectations are reflexive and are seen in her concern for and competency in caring for her infant, in her love and affection for and pleasure in her infant, and [in] her acceptance of the responsibilities posed by the role (p. 374).

Mercer's Becoming a Mother Theory in Nursing Practice .....

According to Mercer (1986, 1995), "the major components of the mothering role are:

1. Attachment to the infant,
2. Gaining competence in mothering behaviors, and
3. Expressing gratification in maternal-infant interactions"

Borrowing from transition theory, Mercer (1995) described the following concepts as having to do with maternal role attainment:

1. pregnancy is a marker event upsetting the woman's status quo,
2. pregnancy requires the woman to move from one reality to another, and
3. pregnancy requires a new role identity"

Mercer's Becoming a Mother Theory in Nursing Practice .....

Mercer (1986) stated that a woman who becomes a mother must do the following:

1. Recognize the permanency of the required change,
2. Seek out information,
3. Seek role models, and
4. Test herself for competency

- According to Mercer the father (or the mother's intimate partner) contributes to the process of role attainment in a way that cannot be duplicated by any other supportive person. As described by Donley (1993), maternal attachment to the infant develops within the emotional field of the parents' relationship.
- The father's interactions help diffuse tension and facilitate maternal role identity. The father's role identity also occurs and increases with infant development and maternal role identity.

Mercer's Becoming a Mother Theory in Nursing Practice .....

- Mercer used a general systems approach in her model of maternal role attainment.
- The mother's microsystem, which is the most influential on her maternal role attainment, includes the mother, her infant, her partner, and intimate relationships within her family.
- The mesosystem includes extended family, school, work, church, and other systems within her more immediate community that directly influence the mother and her microsystem.
- Traits and behaviors influencing maternal role identity are included in the model. Maternal characteristics are empathy or sensitivity to infant cues, self esteem or self-concept, parenting received as a child, maturity and flexibility, attitudes, pregnancy and birth experience, overall health, and role conflict or strain.

Mercer's Becoming a Mother Theory in Nursing Practice

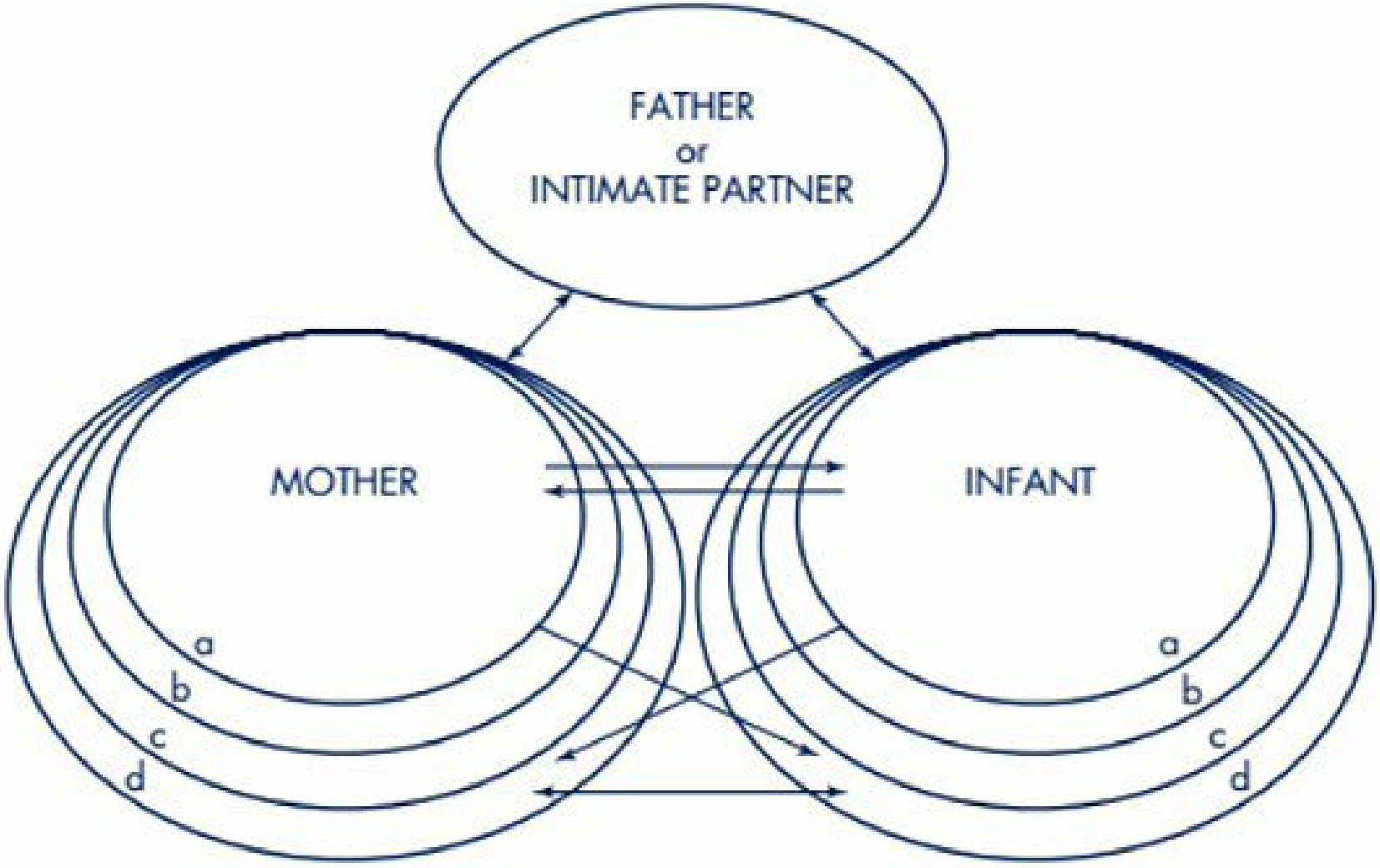


FIGURE 17-1

A microsystem within the evolving model of maternal role attainment. (From Mercer, R. T. [1995]. *Becoming a mother.* New York: Springer.)

Mercer's Becoming a Mother Theory in Nursing Practice .....

A Revised Theory of Becoming a Mother

- Mercer (2004) recognized theory building as a continual process as research provides evidence to clarify concepts, deletions, and additions.
- To summarize the changes in Mercer's revised Theory of Becoming a Mother: Mercer's concept of maternal role identity is unchanged. Mercer (1995) described role identity as having an internal component and an external component. Identity is a woman's internalized view of herself as a mother, and role is the external behavioral component.

Mercer's Becoming a Mother Theory in Nursing Practice .....

- Based on Nelson's (2003) synthesis of qualitative studies, Mercer (2004) changed the names of the stages that lead to maternal role identity using actual words and expressions from studies of women who were in the process of becoming a mother. The changes are as follows:
  - Commitment and preparation (pregnancy)
  - Acquaintance, practice, and physical restoration (first 2 weeks postpartum)
  - Approaching normalization (2 weeks to 4 months after delivery)
  - Integration of maternal identity (approximately 4 months after delivery)

Mercer's Becoming a Mother Theory in Nursing Practice .....

Critical Thinking in Nursing Practice with Mercer's Theory

- According to Mercer (1995), the relationship between the mother and father (or the mother's intimate partner) is of utmost importance. The initial interaction between mother, father, and infant and other immediate family members most often begins in the hospital and can be influenced by nurses.
- For this reason an assessment of the availability of a support person for single mothers or those mothers separated from their mates is also an important consideration for the nurse.
- Nursing goals and patient care outcomes are included under *maternal role identity* and *child outcome* in Mercer's theory of Maternal Role Attainment Nursing care is evaluated based on outcome achievement for maternal role identity and child outcome.

Mercer's Becoming a Mother Theory in Nursing Practice .....

- Factors that influence both maternal and child outcomes are also included in Mercer's original model.
- The following can affect maternal role identity and child outcome: the mother's empathy or sensitivity to infant cues; her self-esteem; the parenting she received as a child; her maturity and flexibility; her attitude; her experiences during pregnancy and birth; and the presence of health, depression, and role conflict.
- Although some factors cannot be changed they are influenced by teaching, guidance, and role-modeling by the nurse.

Conclusion

Mercer's Theory of Becoming a Mother is very useful in assessing, planning, implementing, and evaluating nursing care. The theory is applicable in a wide variety of settings and with diverse populations. Mercer's theory provides not only a framework for practice but also a frame of reference for nursing research. Because of the influence of numerous factors, the process of maternal role identity is complex. Mercer and those building on her work have studied the effect of these factors as variables in research projects, laying the foundation for continued research in this area of nursing to improve practice and the care of women.

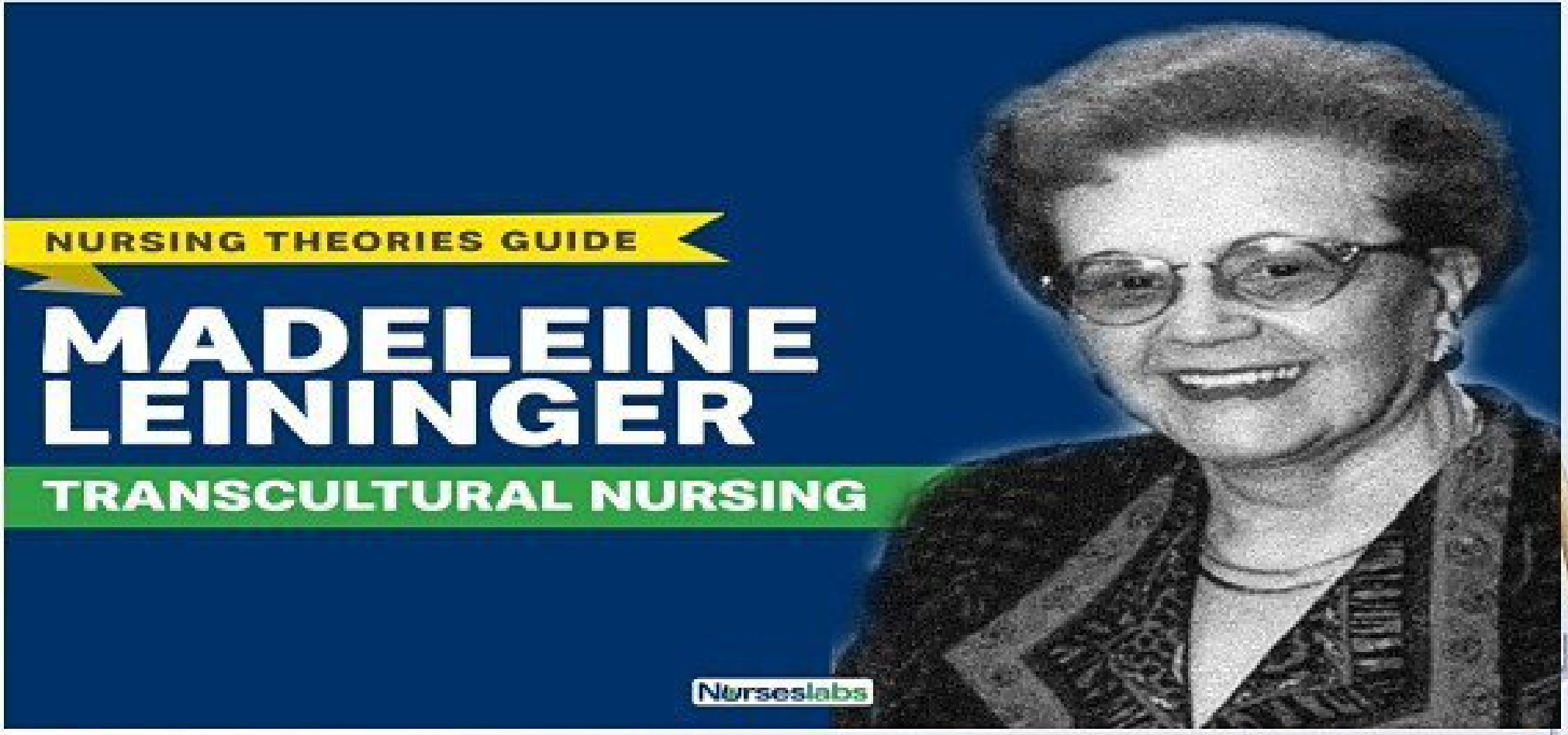
Nursing Theory Utilization & Application

Assist. Prof. Dr. Hala Aiash

2020-2021



Madeleine Leininger



Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice

Nursing is a learned, humanistic, and scientific profession and discipline focused on human care phenomena and caring activities in order to assist, support, facilitate or enable individuals or groups to maintain or regain their health or wellbeing in culturally meaningful and beneficial ways, or help individuals face handicaps or death.

(Leininger, 2002a, p. 46)

History and Background

- Nurse anthropologist Madeleine Leininger developed the culture care theory and ethnonursing research method to help researchers study transcultural human care phenomena and discover the knowledge nurses need to provide care in an increasingly multicultural world.
- She realized that she needed to go beyond anthropology with its emphasis on groups of people in different parts of the world to express her thoughts from a nursing perspective. At that time nursing practice was based on the medical model and nurses practiced primarily in hospitals or public health departments.

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

- Leininger (1995a) stated, "I envisioned that transcultural nursing was different from anthropology in that the focus was on comparative health care, health, and well-being in different environmental contexts and cultures"
- Before Leininger founded the field of transcultural nursing she believed that care was the most important component of nursing. She stated, "Care is the essence and the central, unifying, and dominant domain to characterize nursing"
- Leininger (2002a,b, 2006, 2007, 2011) continued throughout her life to clarify the essential features of culture care diversity and universality theory within the context of transcultural nursing

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

- Transcultural nursing is a substantive area of study and practice focused on comparative human care (caring) differences and similarities of the beliefs, values, and practices of individuals or groups of similar or different cultures.
- Leininger (2011) reported that culture care patterning and/or restructuring actions and decisions were very difficult for fathers in the four cultural groups to consider as they wanted to preserve their cultures and traditional care practices. However, all groups reported that some members embraced modern technologies but feared harm from their use.
- Her legacy is the theory of culture care diversity and universality and transcultural nursing that continues to inspire those whom she mentored, taught, and influenced throughout her accomplished career.

Overview of Leininger's Culture Care Theory

- The construct of culture in Leininger's theory borrows its meaning from anthropology.
- Culture is the "learned, shared, and transmitted knowledge of values, beliefs, norms, and lifeways of a particular group that are generally transmitted inter-generationally and influence thinking, decisions, and actions in patterned or certain ways"
- Culture can be discovered in the actions, practices, language, norms or rules for behavior (values and beliefs), and in the symbols that are important to the people. As Leininger has stated, culture is learned and then passed down from generation to generation.

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

- The goal of the culture care theory (CCT) is to provide culturally congruent nursing care, which refers to "culturally based care knowledge, acts, and decisions used in sensitive and knowledgeable ways to appropriately and meaningfully fit the cultural values, beliefs, and lifeways of clients for their health and wellbeing, or to prevent illness, disabilities, or death"
- The culturalogical assessment consists of a comprehensive holistic overview of the client's background including communication and language, gender and interpersonal relationship customs, appearance, dress, use of space, food preferences, meal preparation, and other lifeways.

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

- Leininger's theory is applicable in the nursing care of clients from racially and ethnically diverse backgrounds as well as the culture care needs of individuals or groups belonging to cultures and subcultures identified on the basis of sexual orientation ability or disability , occupation, age, or socioeconomic status.

1. Two other constructs of importance in the theory of culture care diversity and universality are culture-specific care and culturally congruent care. Culture-specific care refers to care resulting from the identification and abstraction of care practices from a particular culture that lead to the planning and application of nursing care to "fit the specific care needs and life ways" of a client from that culture.

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

2. Culturally congruent care is a major goal of the theory (Leininger, 2006). This refers to "culturally based care knowledge, acts, and decisions used in sensitive and knowledgeable ways to appropriately and meaningfully fit the cultural values, beliefs, and practices of clients for their health and wellbeing, or to prevent illness, disabilities, or death"

- Leininger (2002b) postulated three modes of care actions and decisions for guiding nursing care. The three modes of culture care are (a) preservation and/or maintenance; (b) accommodation and/or negotiation; and (c) patterning and/ or restructuring.
- Leininger's culture care theory provides practicing nurses with an evidence-based, versatile, useful, and helpful approach to guide them in their daily decisions and actions regardless of the number of clients under their care or complexity of their care needs.

Critical Tinking in Nursing Practice with Leininger's Theory

Leininger's (2002a, 2007) theory of culture care diversity and universality guides practice by assisting nurses to be culturally aware of and sensitive to individual cultures; then to groups and families; then to institutional, regional, and community, societal, and national cultures; and eventually to global human cultures. The sunrise enabler guides decisions and nursing actions through a process focused on specific components of the theory. (Figure 18-1).

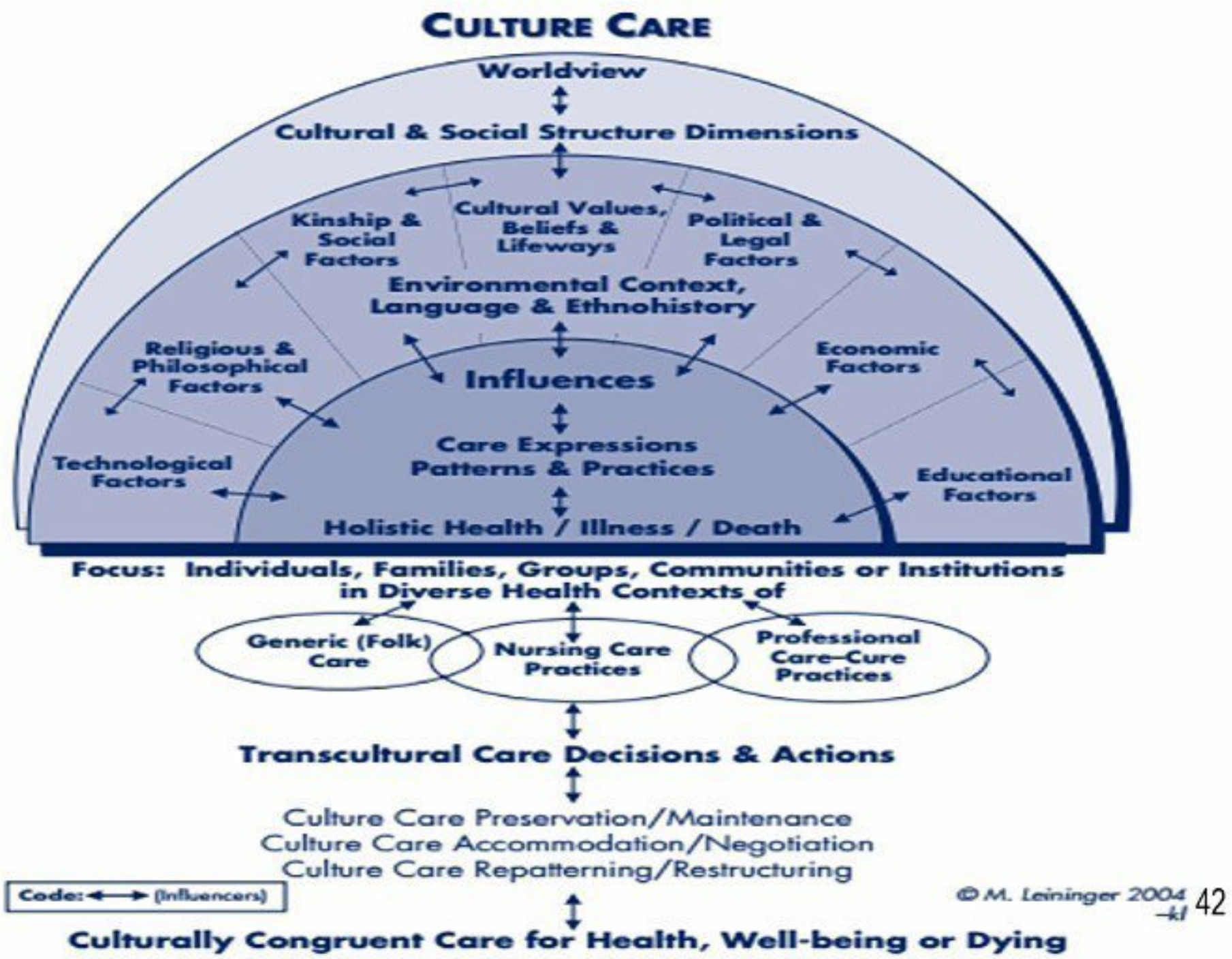


FIGURE 18-1

Leininger's sunrise enabler to depict the Theory of Culture Care Diversity and Universality. (Copyright © Madeleine Leininger. Modified by Madeleine Leininger, 2004.)

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

- The sunrise (model) enabler was revised and renamed enabler to clarify it as a visual guide for exploration of cultures. As Leininger (1995d) stated, "This model should not be viewed as a theory per se, but rather as a depiction of the multiple components of the theory"

- In most cultures the family is an important factor in care. She used the three modes from the CCT to describe her discovery that the provision of culturally congruent care was centered around the family. Culture care preservation was maintained by requesting that nurses avoid pressuring relatives of deceased Muslim patients to give consent for organ donation or autopsy because Muslims believe their bodies are a gif from God and themselves as trustees of this gift.

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

- The nurse practitioner needs to be able to sensitively and competently integrateculture care into contextual routines, clinical ways, and approaches to primary care practice through role-modeling, policymaking, procedural performance and performance evaluation, and the use of the advanced practice nursing process.
- McFarland and Eipperle (2008) list six criteria for theory application in advanced practice nursing:

1. Be inclusive rather than exclusive

2.Foster a focus on the whole person rather than the disease or illness.

3. Include consideration of the patient's/family's/signifcant other's perception of the situation.

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice .....

4. Be holistic in nature which is helpful to both practice and documentation.
5. Facilitate autonomous nursing practice (aspect of professionalism along with knowledge and service).
6. Encourage diverse ways of knowing, including empirics, ethics, aesthetics, personal knowing, and sociopolitical knowing.

- As has been noted, "Healthcare organizations should be committed, willing, and supportive of their staff to ensure the attainment of a culturally competent work environment" (McFarland & Eipperle, 2008, p. 50).

Leininger's Theory of Culture Care Diversity and Universality in Nursing Practice …….

- Mixer (2011) used both the culture care theory and the ethnonursing research method to conduct a unique study of nursing faculty to discover the complex nature of teaching culture care to nursing students.

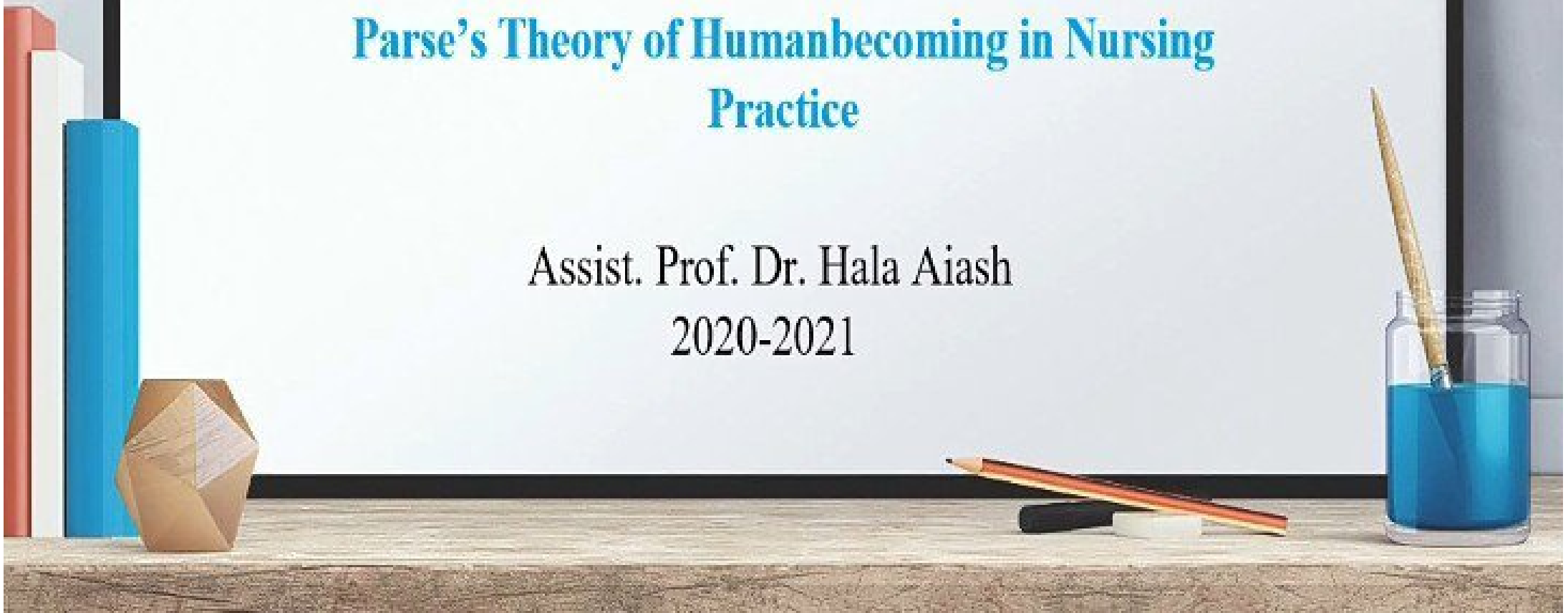
- Mixer (2011) discovered preserving faculty generic care based on their religious values, beliefs, and practices would enhance teaching culture care. As faculty participated in this study, it became clear to them that teaching culture care was given minimal attention and was not integrated throughout their curricula.

Nursing Theory Utilization & Application

Parse's Theory of Humanbecoming in Nursing Practice

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2020-2021



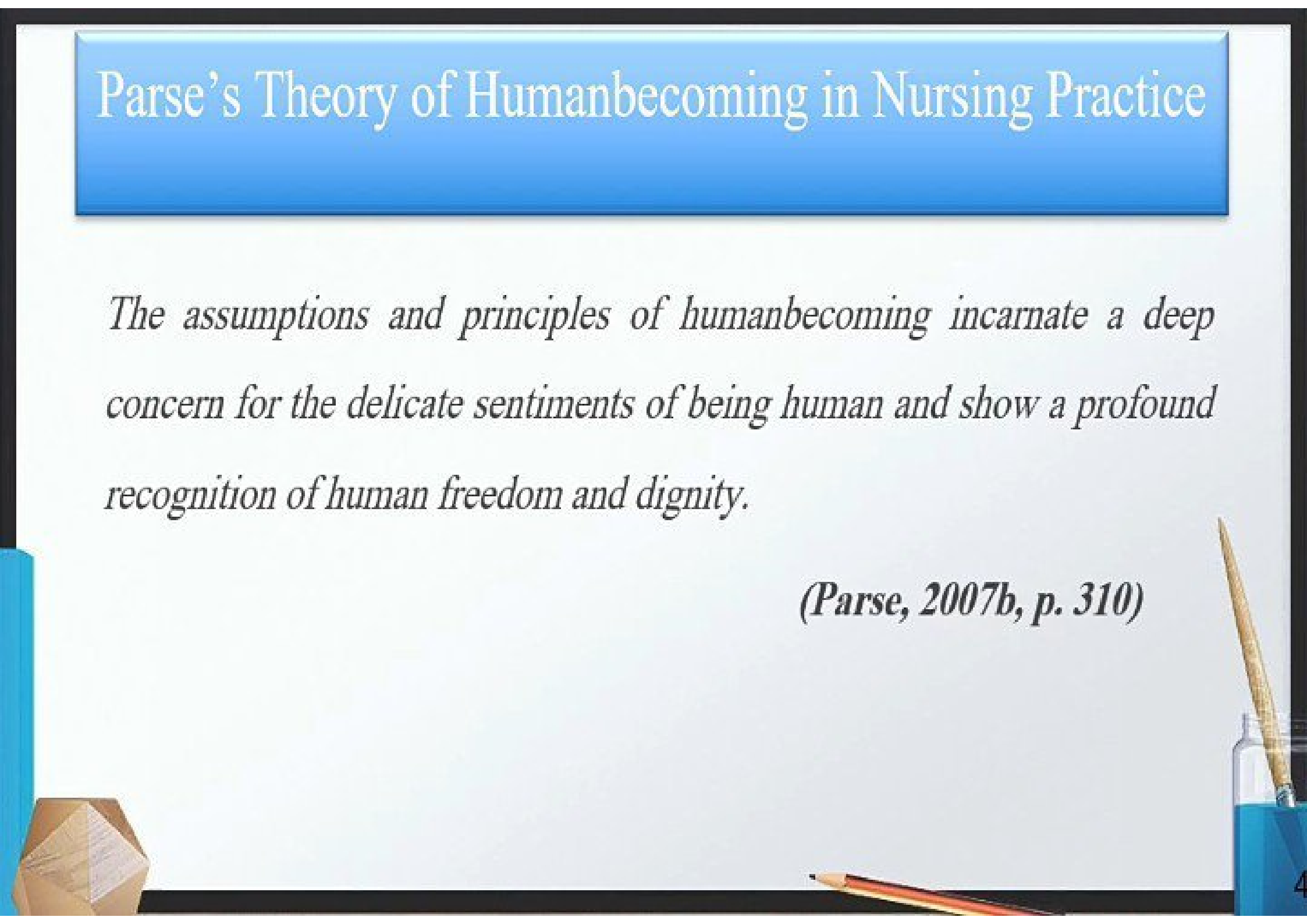
Rosemarie Rizzo Parse



Parse's Theory of Humanbecoming in Nursing Practice

The assumptions and principles of humanbecoming incarnate a deep concern for the delicate sentiments of being human and show a profound recognition of human freedom and dignity.

(Parse, 2007b, p. 310)



Parse's Theory of Humanbecoming in Nursing Practice .....

History and Background

- In the video series *Portraits of Excellence,* Parse (1990b) shared that the humanbecoming theory, now an aspect of the Humanbecoming School of Thought (Parse, 1998, 2007b, 2012a,b), was created over many years as she grew and matured with others and as she considered the values and ideas she engaged while living and learning.
- Parse found the medical model limiting and inconsistent with her experience of how people make health decisions. Accordingly, she searched for a different way to know and practice nursing. From the beginning, Parse chose to focus on people's experiences and their unique views of health. She believes that people co-author their health, and she asserts that nurses do not have control of people or of their health choices.

Parse's Theory of Humanbecoming in Nursing Practice .....

- The nursing science she envisioned is consistent with basic tenets of the human science tradition and supports practices that truly honor the freedom and dignity of human beings.
- Parse's work, originally called *man-living-health,* appeared in print in 1981. The name was officially changed to *human becoming* in 1992 (Parse, 1992a) and then to *humanbecoming* in 2007.
- For Parse (1998, 2007b, 2012b), humans are indivisible, unpredictable, and everchanging, and they make choices while living paradoxical patterns of becoming.
- The Theory of Humanbecoming itself, which consists of three principles and nine concepts, was developed in deductive-inductive ways during the 1970s as Parse pondered the possibilities of a different kind of nursing.

Parse's Theory of Humanbecoming in Nursing Practice .....

- Parse (2012b) introduced new conceptualizations in 2012 that further specify the meaning of the all-at-onceness of human experience from a humanbecoming perspective.
- Currently, within the humanbecoming school of thought, are assumptions; postulates; principles; concepts; practice and research methodologies; models of teaching-learning, mentoring, leading-following, family, and community and ethical tenets of human dignity.
- The phrase *school of thought* is more comprehensive than the humanbecoming theory itself, and development of methodologies, models, and ethical tenets indicates the potential of the belief system to support study, knowledge development, and work in multiple arenas.

Overview of Parse's Theory of Humanbecoming

With humanbecoming, health cannot be given or taken, controlled or manipulated, judged, or diagnosed. Rather, health is the way people live their values in accordance with their desires, hopes, and dreams.

From Parse's perspective, individuals co-author their health and no outsider can define the values that another person will cherish and live by. This view of health, along with the assumptions and principles of humanbecoming form the foundation for the kind of practice directed by the theory.

Parse's Theory of Humanbecoming in Nursing Practice .....

The humanbecoming theory consists of three principles and nine concepts that flow from assumptions about humans and becoming. The nine assumptions (Parse, 2012b) are as follows:

1. The human with universe is co-existing while co-constituting rhythmical patterns.
2. The human is open, freely choosing meaning with situation, bearing responsibility for decisions.
3. The human is continually co-constituting patterns.
4. The human is transcending illimitably with possibles.
5. Becoming is human-living-health.
6. Becoming is rhythmically co-constituting humanuniverse.
7. Becoming is the human's value priority patterns.
8. Becoming is transcending with possibles.
9. Becoming is the human's emerging.

Parse's Theory of Humanbecoming in Nursing Practice .....

Three themes emerge from the nine assumptions: meaning, rhythmicity, and transcendence. Three principles of humanbecoming (the theory) flow from the themes (Parse, 1981, 1998, 2007b, 2012b):

1. Structuring meaning is the imaging and valuing of languaging.
2. Configuring rhythmical patterns is the revealing-concealing and enablinglimiting of connecting-separating.
3. Co-transcending with possibles is the powering and originating of transforming.

The humanbecoming theory is unique in its identification of paradox as an inherent process of human experience. The three principles of humanbecoming form the knowledge base that shapes the nurse's view of humanuniverse.

Critical Thinking in Nursing Practice with Parse's Theory

The ability to reflect on and define coherence between knowledge and action is thus an indicator of critical thinking. Parse (1996a) states that "critical thinking…is carefully choosing a direction in light of personal tacit and explicit knowing…and choosing a direction is moving beyond the moment with deliberateness"

A nurse guided by humanbecoming documents paradoxical patterns that a person expresses and the desired directions/actions identified by the person. The desired change or goal is defined by the person rather than by the nurse. The nurse's responsibility is to record the person's evaluation, ongoing concerns, preferred choices, and meanings as they surface in practice.

Nursing Theory Utilization & Application

Newman's Theory of Health as Expanding Consciousness in Nursing Practice

Dr. Hala Aiash

2020-2021

Margaret Newman



Newman's Theory of Health as Expanding Consciousness in Nursing Practice

Nurses practicing within this perspective experience the joy of participating in the transformation of others and find that their own lives are enhanced and transformed in the process of the dialogue.

(Newman, 2008, p. 29)

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

History and Background

- The first published version of Newman's theory appeared in her book *Theory Development in Nursing* (Newman, 1979). In this early writing, Newman presented a viewpoint of health as a dialectic fusion of disease and non disease, thus encompassing disease as a meaningful aspect of the totality of life experience.
- Evolution of the conceptualization of Health as Expanding Consciousness (HEC) was influenced theoretically by several sources.

*Bentov's (1977) explanation of the evolution of consciousness,

*Moss's (1981) view of love as the highest level of consciousness,

*de Chardin's (1959) belief that consciousness continues to develop beyond physical life.

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

- The Theory of HEC evolved from a synthesis of these theoretical influences with Newman's own life experiences and thoughts.
- Newman explored effects of changes in rates of walking on time perception in two studies and the relationship between age, movement, and time perception of elders in another study.
- The professional nurse is viewed as a therapeutic partner who joins with the client in the search for pattern with its accompanying understanding and impetus for growth.
- Newman (1990a) developed a practice methodology of pattern identification and research/practice process. This method reveals sequential patterns of persons' lives and facilitates recognition and insight into patterns.

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

- The theory research or practice application has been used with cancer patients elderly women with chronic disease black-Caribbean women persons with coronary heart disease human immunodeficiency virus/acquired immunodeficiency syndrome (HIV/AIDS).
- Pharris (2002, 2005) advocated application of the theory to individuals, families, and communities. The theory has been applied to nursing management.
- Recent global use of Newman's theory is reflected in publications by nurses worldwide.

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

Overview of Newman's Theory of Health as Expanding Consciousness

- In the early development of the theory Newman asserted that the phenomena of inquiry for nursing should be parameters of human wholeness and that there were characteristics of people that identified the whole (Newman, 1979).
- Time, space, and movement were identified by Newman as dimensions of pattern and consciousness and have been synthesized as the theory evolved to include the major concepts of health, consciousness, and patterns of movement and space time.

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

Health

- In Newman's theory, health is an expansion of consciousness that is defined as the informational capacity of the system and is seen as the ability of the person to interact with the environment .
- Disease is a meaningful reflection of the whole and, as such, is viewed as a manifestation of health
- According to Newman "Health is the pattern of the whole, and wholeness is. One cannot lose it or gain it"

Consciousness

- Consciousness is defined in the theory as the informational capacity of the system (human beings) or the system's ability to interact with the environment .
- Consciousness includes not only cognitive and affective awareness but also the "interconnectedness of the entire living system, which includes physiochemical maintenance and growth processes as well as the immune system"

Pattern

- Pattern has dimensions of movement and space-time. It is constantly moving unidirectionally and evolving and may be enfolded in a larger pattern that is in the process of unfolding. Using Rogers' (1970) conceptualization of pattern, Newman (1986) states, "Pattern is information that depicts the whole, understanding of the meaning and relationships at once. It is a fundamental attribute of all there is and gives unity in diversity"
- Time, as a dimension of pattern, is conceptualized as either subjective or objective and also is viewed in a holographic sense.
- Further, time is considered an index of consciousness (Newman, 1983) because as consciousness expands, space-time transcends limitations of linear and physical boundaries to extend beyond what is the here-and-now.

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

- The dimensions of space and time are complementary and inextricably linked to each other as space-time or time-space, with time being increased as one's life space decreases (Newman, 1979, 1983). Space has further been identified as life space, personal space, and inner space ,with personal space or territory very much involved in a person's struggles for self-determination and status .
- According to Newman (1994a), movement is a reflection of consciousness, indicates inner organization or disorganization of people, and communicates the harmony of a person's pattern with the environment.

Critical Tinking in Nursing Practice

- From Newman's perspective, nursing is the study of "caring in the human health experience" .Newman emphasizes that the essence of the process is to be fully present in the transformation of clients and ourselves as we search for meanings in the lives of those who come to us at critical junctures of their lives .
- The responsibility of the professional nurse is to establish a primary relationship with the client for the purpose of identifying meaningful patterns and facilitating the client's action potential and decision-making ability.
- The relationship is one of rhythmicity and timing. The nurse surrenders the need to direct the relationship or to "fix" things. As the nurse relinquishes the need to manipulate or control, there is a greater ability to enter into this fluctuating, rhythmic partnership with the client.

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

- Newman (2008) has outlined the process (praxis) that involves a merging of the a priori Theory of HEC, research, and practice. The following steps are included in the process:

1. Engagement with the client/participant (CP), in which the emphasis is on the unfolding pattern of the CP's life.

2. Development of the narrative, in which the nurse notes the most important statements of the CP to form a trajectory of the most significant relationships and identifies sequential patterns over time, writing and diagramming the patterns
3. Participation in follow-up meeting(s), in which the nurse shares the data with the CP without interpretation for the CP to confirm or revise and insight occurs

Newman's Theory of Health as Expanding Consciousness in Nursing Practice .....

4- Application of the theory, in which, although the theory is active throughout the encounter, the nurse uses post-encounter analysis in relation to the Theory of HEC and notes transformational changes.

5.Identification of reciprocal family and community patterns.

- Nurses practicing within the Newman framework are involved in a relationship process that co-evolves as a function of the interpenetration of evolving fields of the nurse, client, and environment in a self-organizing and unpredictable way.